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Morris County Prosecutor (6/2007-2/2013)

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June 26, 2019

Via E-file

The Honorable Angela Borkowski, J.S.C.
Hunterdon County Superior Court
65 Park Ave.
P.O. Box 756
Flemington, NJ 08822-0756

RE: State v. Steven Bradley Mell, (a.k.a Brad Mell)
Complaint No. W-2018-000117-1022

Dear Judge Borkowski:

We respectfully submit this Memorandum on behalf of our client Steven Bradley Mell in connection with his sentencing proceeding scheduled for June 28, 2019.

On May 13, 2019, this Court received Mr. Mell's plea of guilty, where Mr. Mell pled guilty to the following offense: (1) 2C:24-4a(1), Endangering the Welfare of a Child a crime in the 3rd degree.

The Defendant's sentence is to run concurrent with the Defendant's Federal sentence under (3:18-Cr-00757-BRM) of 84 months in prison with 5 years of supervised release, and the New Jersey State sentence of 3 years must be stayed until after the Mr. Mell voluntarily surrenders to his Bureau of Prisons designated facility.

PRELIMINARY STATEMENT

Mr. Mell's decision to cooperate and take full responsibility for his own conduct reflects his personal resolve, notwithstanding past errors, to re-point his internal compass true north toward a productive, ethical and thoroughly law-abiding life.

We now turn to the additional factors relating to sentencing. Our comments below, respectfully, are intended to be taken in the above-expressed spirit, in which Mr. Mell has agreed to accept responsibility. He accepts full responsibility for the offense conduct, and we, as counsel, submit the below considerations in this context.

Mr. Mell's Personal History and Characteristics

Mr. Mell was born on December 15, 1965 at Morristown Memorial Hospital in New Jersey. He has two siblings, Courtney and William, who are 54 and 55 respectively. His mother, Diana age 81, thankfully is still alive in South Carolina; and his father, William Harvey, passed away in December of 2008.

Mr. Mell was raised in a loving and compassionate family. He demonstrated to them early on that he had the drive and focus to succeed, and that he was determined to dedicate all his time and energy to his family, his charity, his work, his employees, and his hobbies. Indeed, it is his care and compassion for others that has impacted the lives of so many people, as stated by his mother, Diana Mell.

My son is a warm, considerate, thoughtful and generous person and has never given his family one ounce of trouble or hardship...He is loved by his loved his brothers and children, who stand behind him and support him unconditionally.

Brad is a well-respected businessman and individual, always putting the needs of others before his own. [Exhibit A1]

Mr. Mell's academic career began in New Jersey at Far Hills day school from K-9th grade, Tilton Prep School in New Hampshire for High School, and Boston University for college where he graduated in 1988 with a major in psychology.

Early on, Mr. Mell struggled in school barely making C's and D's. Fortunately, Headmasters of Far Hills Day School, Russ Ball and Chuck Scranton, recognized Mr. Mell was suffering from dyslexia. When he was diagnosed with an extreme case of dyslexia, he worked tirelessly to overcome his obstacles. Indeed, it paid off. In seventh-grade, Mr. Mell went from C's and D's to an improvement so drastic that he received the Headmaster's Award.

After attending Far Hills Day School, he enjoyed the opportunity to attend Tilton Prep School in New Hampshire. He excelled at Tilton, participating in three sports including the varsity hockey team. Along with his performance as an athlete and scholar, he also found time to participate in theater and in his senior year he was honored with the Theater Award for his work on the productions of the school's plays.

As a teen, Mr. Mell became passionate about flying. His desire to always put his focus and energy into everything he did was never more apparent than his love for flying. He received his pilot's license at the age of 16, and in 2008, he received his fixed wing and helicopter rating. His love of flying, along with the peace, serenity, and joy he got from being in the air became the source of his greatest memories.

In 1984, Mr. Mell moved to Boston where he attended college at Boston University. Mr. Mell was always actively looking to serve, so in 1987 he became an auxiliary police officer in Boston. In 1988, Mr. Mell graduated college with a M.A. in Psychology. During the next few years, Mr. Mell worked as a trader for the State Street Bank in Boston and began to develop a

talent for finance. It was also during this time that he found his greatest love- Kimberly Ruggles, whom he married in 1992.

Mr. Mell and his wife moved back to New Jersey, where Mr. Mell began his new career with his father at W.H. Mell Associates. He started out as a municipal broker dealer making less than \$20,000 per year. He hoped that he would be able to grow W.H. Mell Associates into the business through his tireless work and aptitude, but Mr. Mell being who he is did not just grow into the business; he helped transform the company from the ground up. In 1996, Mr. Mell took over as company President, and as President of W.H. Mell, Mr. Mell was responsible for the continued employment and support of over 15 associates and their families. His employees have always been like family to Mr. Mell, and he continued to grow the business of the firm to staggering heights.

Gratefully, it was during this time that Mr. Mell also built his life as a proud family man. Mr. Mell's first daughter, Emily, was born in 1996. She graduated from Bucknell University with a degree in education the weekend after his arrest. Mr. Mell was forced to miss the graduation due to his detention which will be a regret until the day he dies. In 1998, Mr. Mell's second daughter, Eliza, was born. She is now attending Colby College as a pre-med veterinary student, and, in 2003, Mr. Mell's son, William, was born. He is a high school student at Westminster School in Connecticut.

Mr. Mell loves and is proud of all his children. Even while running an organization of multiple companies, Mr. Mell always had time for his children. He has been integral in the education of his children, constantly being present and active for every event, play, graduation, conference and game. Mr. Mell became an active member of school boards and hockey teams. He enjoyed three years with Eliza taking her around the country playing travel hockey with the

Colonials. He was a coach of the hockey team for over 10 years at the Essex Hunt Club. And for 15 years, Mr. Mell served as a board member at Westminster School, the last two years of which he also served as Chair of the Investment Committee.

Incredibly, Mr. Mell also devoted time and service on the Far Hills Day School board for 10 years as a way to give back to the school that helped him overcome his extreme dyslexia. He served for over 10 years as the Chairman for Building and Grounds. In 2017, Mr. Mell was celebrated with a special dedication for his work during his 10-year volunteer service to the school. Wendy Cutler, friend of Brad since childhood acknowledges this event, “He received the “Unsung Hero Award” at Far Hills Country Day School for quietly and anonymously donating his time and financial resources to the school.” **[Exhibit A2]**

Life seemed to be going great for Mr. Mell. He had a wonderful family, a successful business, and an active community life. It seemed that nothing could ever go wrong, until October 1, 2011: Mr. Mell was scheduled to attend a golf outing with friend Will Ellsworth, and two others in New York. Will, who he considered his best friend, planned to travel via helicopter to the Pine Valley Golf Club and return the same evening, but it was a last-minute work circumstance that caused Mr. Mell to back out of the trip and ask his friend Tighe Sullivan to take his place.

Tragically, Will Ellsworth and Tighe Sullivan while traveling in one of Mr. Mell’s helicopters crashed the very same day just outside of Mount Pocono, both his friends died in the crash. The survivor’s guilt sent Mr. Mell into a spiral of depression. He was burdened with the thought of their families and guilt for the suffering his own family avoided.

Mr. Mell sought the help of a therapist. Coping with the loss of friends – two of whom died on that helicopter- has been an arduous process. But it was in therapy that Mr. Mell was able to see how much he was doing for so many people, and how the burden of his responsibility to so

many people was taking its toll on him emotionally. He had supported his father and mother through his father's alcoholism and continued to pay all the bills for his parents as well as some of his siblings. His responsibility to his family was enormous, and Mr. Mell never balked at the opportunity to give to so many.

Mr. Mell has not only been generous with his family and his employees, but he knows that providing and caring for others who are not as fortunate as himself is vital to the community. William, his brother, writes about the selfless philanthropic nature of his brother.

Another great example of this is that Brad does Air Lifeline which is now known as Angel Life flights for people that need better medical care in a different part of the county so he either flies them himself to wherever or, at this own expense, has a pilot use his plane to get the patient to wherever they need to go. [Exhibit A3]

While Mr. Mell helped to build a successful business, organized numerous charitable programs, and achieved numerous awards as a student, coach, board member and businessman, he prefers not to live in the spotlight but rather to let his actions as a husband, father, business owner, and volunteer be the story of his life. As a humble man, Mr. Mell has not announced the accolades he has received in his lifetime. Instead, his family and friends proudly proclaim the impact Mr. Mell's actions and service has had on all of them.

Lifetime Contacts and Charitable Foundation

Mr. Mell has a lifetime of contacts in the New Jersey area, many of whom are well-respected professionals. He has always been well known in the community as an upstanding professional holding several licenses in finance, including his series 63, 7, 52, 24 and 53. He is longstanding board member on the Essex Hunt Club, Tilton Prep School, Somerset Hills Country Club and was a member Far Hills Day School Board from 2004-2014.

Mr. Mell started his own charitable foundation, Air Lifeline (merged with Angel Flights in 2009), that has donated to families of children with medical needs, the ability to fly anywhere in the country in order to received medical treatment.

Alan Sobel, certified public accountant and friend of Mr. Mell for over twenty years recalls that, "...Brad was a dedicated husband, father, and friend. Family and friends were the most important things in his life. He was incredibly charitable in his giving, both time and money, to the community he lived in and the schools his children attended." **[Exhibit A4]**

Mr. Mell is a donor to the Mooreland Foundation, Morristown Memorial Hospital, NJ Visiting Nurse Association, Far Hills Day School, Westminster School, Red Cross, and Brady Life Camp (a Newark inner-city kids camp). He has given to countless other charities choosing to remain anonymous in his generosity.

Mr. Mell's incredible generosity and love for his friends shines through in a letter from Perry Hall, a friend of Mr. Mell's since childhood:

My father died in July 2012 and we planned the funeral shortly thereafter. It was in the middle of summer and many of my friends were on vacation and busy with their own young families. Entirely on his own, Mr. Mell arranged to fly about 8 of my closest friends to attend the service and return them back to their own families the same day so as not to disrupt family vacations. While Mr. Mell knows these folks through mutual friends, these were my closest college friends – not his. He knew how much the effort would mean to me at a very difficult time. He didn't ask anyone to help pay and wasn't doing it for any kind of personal gain, but rather because he knew how great it would be for me to have such a strong support network. **[Exhibit A5]**

Mr. Mell has up to this point in his life led a life free from criminal prosecution and is considered by his friends and family as an asset to his community. Mr. Mell has consistently demonstrated throughout his life his accountability and integrity to his family, his business, his

employees, his clients, and the community. His invitation to sit on multiple school boards, local clubs and organizations, along with his 25-year stellar track record with his clients further shows that he is a man who is trusted by many respected members of our community.

Health

In 2004, Mr. Mell was diagnosed with a life-threatening illness of Sclerosing Cholangitis and Crohn's Disease. Sclerosing Cholangitis is a chronic liver disease of the bile duct and was a told he could expect another 8- 10 years of life. That was 16 years ago, and Mr. Mell reports living for 10+ years with the palpable sense that he could die at any time. He is grateful to have been blessed with the additional years and has tried to make each moment that he has spent with his family that much more lasting and memorable.

While his diagnosis, does not diminish the pain that he has caused so many others, he hopes that he will be able to have more moments with his children and family beyond this sentence.

Evaluations

Mr. Mell was previously evaluated by Dr. Catherine Barber, Ph.D. as appointed by United States Pre-Trial Department, on May 31, 2019 as well as Dr. Richard Krueger on September 19, 2018. Each doctor has determined that Mr. Mell is a low risk for re-offense and the circumstances related to the offense are not likely to reoccur.

...Mr. Mell's score of 0 places him in the "below average risk" range. Among a sample of routine sex offenders (non-high risk), between 2.2 and 3.5 out of 100 offenders with a score of 0 would be expected to be charged or convicted of new sexual offense by five years post-release. [**Exhibit B1**]

Mr. Mell is not a pedophile and does not have any pattern of paraphilic or hypersexual behavior. His risk of a future sexual crime is low or very low according to 5 instruments used to assess such risk. [**Exhibit B2**]

Recently, on April 9, 2019, Mr. Mell was re-evaluated by Dr. Richard Krueger prior to his federal sentencing. This re-evaluation included another interview of Mr. Mell and additional objective testing. Dr. Krueger's opinion remains unchanged, "Mr. Mell is not a pedophile and does not have any pattern of paraphilic or hypersexual behavior. His risk of a future sexual crime is low or very low according to 6 instruments used to assess such risk and will be reduced by both therapy and supervision when he is in the community." [Exhibit B3]

Furthermore, in a report authored by Dr. Daniel Watter, who has been Brad's therapist since mid-August 2018 has agreed to author a report that details the circumstances that may have led to choices that Mr. Mell made and thus leading to current criminal action against him. "Mr. Mell's behavior was, by all accounts, *uncharacteristic* of any of his prior behaviors. This point is particularly salient when assessing the factors that were driving Mr. Mell's inappropriate sexual behaviors. It is for this reason, as well as his traumatic family history, that I characterize Mr. Mell as a trauma patient as opposed to a sex offender." [Exhibit B4].

Letters of Support and Acceptance of Responsibility

Despite being charged and pleading guilty to the crimes so charged, Mr. Mell has continued to receive support from his family and friends. Here contained are additional letters of support for Mr. Mell which acknowledge that the incident contained within the State's complaint is an aberration, inconsistent with the man that they have raised, grow-up with, and considered a friend.

Lt. Col Hunter Kellogg and his wife write about Mr. Mell's acceptance of responsibility. "We are aware of Brad's wrongdoings, and acknowledge their seriousness, but also his acceptance of responsibility. He has offered no excuses, only context." [Exhibit A3]

Additionally, Wendy Cutler discusses the man she has known and his contrition for this act. "Brad is so remorseful, so sad, so disheartened and so apologetic for his actions. He knows the damage he has caused to his family and that is going to be very difficult to make

amends...please don't judge him solely on this one chapter of his life. Throughout his life, Brad has performed countless, selfless acts of kindness." **[Exhibit A4]**

Joy Frelinghuysen friend since middle school, gives an honest assessment of the choice Brad made as well as the love he has for his family and how he has their support and love.

These loving, supportive aspects of Brad's character found their true north when he had children. In fact, his most beautiful quality is his deep love for his children. Do I realize the irony of this statement given the circumstances of his case? I absolutely do. For all of us who know and love Brad, his poor judgement and choices in this matter will never make sense, but here is what I can tell you for sure. His three children do not have one doubt in their minds about his undying love for them. **[Exhibit A7]**

Additionally, Mr. Mell takes, "full responsibility for my conduct in this case. I am embarrassed and ashamed that I permitted myself to cross the line and violate the law. I am prepared to accept the Court's prison term, do my time in prison, and move forward making positive contributions to society and being the best possible father possible. I want to apologize to [Minor Victim 1] I am sorry for my actions and it is my hope that everyone can move forward with this difficult period of their lives."¹

Mr. Mell is blessed with family and friends who are aware of the ramifications of his guilty plea and the loss his children will experience without him around for the next 84 months. However, they know that he must atone for his actions and that when he is released, he will continue to be the productive and giving man he was before this incident.

¹ June 19, 2019 Presentence Report, Hunterdon County – *Attachment 1*

CONCLUSION

Accordingly, for all the reasons stated above, and based on all the sentencing factors the Defendant humbly requests that this Honorable Court accept the terms of Defendant's plea, and sentence Mr. Mell to a prison term of 3 years to run concurrent to the federal sentencing, with the primary custodial sentence be served within the Federal Bureau of Prisons designated facility.

EXHIBIT A1

The Bianchi Law Group, LLC
4 York Avenue, 2nd Floor
West Caldwell, New Jersey 07006

For Scanned/Emailed Letters:
rsesnes@bianchilawgroup.com

Mrs. Diana Mell
72 Fletcher Hall
Kiawah, SC, 29455

March 18, 2019

The Honorable Brian R. Martinotti
United States District Judge for the District of New Jersey
Clarkson S. Fisher Federal Building & US Courthouse
402 East State Street
Trenton, New Jersey 08608

Re: United States of America Vs. Steven Bradley Mell

Dear Judge Martinotti,

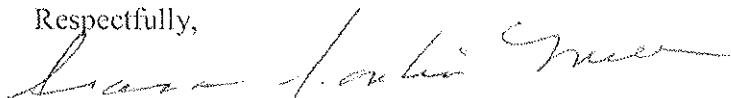
I am Diana Donlin Mell and am Steven Bradley Mell's ("Brad") natural and biological mother. I have raised Brad from birth and continue to have a very close relationship with him

I am acutely aware of Brad's guilty plea and wish to provide Your Honor with my assessment of this as an aberration from the son that I know and love. My son is a warm, considerate, thoughtful and generous person and has never given his family one ounce of trouble or hardship. He has always been a very attentive son and has truly been a God-send to me. He is loved by his brothers and children, who stand behind him and support him unconditionally.

Brad is a well respected businessman and individual, always putting the needs of others before his own. He is active in his community, has been on the boards of Westminster Schools, Far Hills Country Day Schools, and also coached girl's hockey at the Essex Hunt Club. Brad also founded Aero Care Life Flights, which is an organization that provides free air transportation for those that need medical treatment far away from their home and also transportation for other community services. He has often provided his own private aircraft for use by families who have lost loved ones.

I humbly request leniency and mercy from the Court in sentencing for my son and request that the Court follow the terms of the plea agreement which states that Brad will be sentenced to federal prison for 60 months. Thank you for your time and consideration.

Respectfully,



Diana Donlin Mell

EXHIBIT A2

Wendy Cutler
110 Bowery Road
New Canaan, CT 06840

March 21, 2019

The Honorable Brian R. Martinotti
United States District Judge for the District of New Jersey
Clarkson S. Fisher Federal Building & US Courthouse
402 East State Street
Trenton, New Jersey 08608

Re: United States of America vs. Steven Bradley Mell

Dear Judge Martinotti,

Brad has been an integral part of my life since we were toddlers. In fact, one of my mother's favorite pictures is of the two of us in diapers. Brad and I went to elementary school together through junior high. Most days, after school, Brad logged countless hours at my house. It is fair to say my mother was his "second mom" I think he was even given chores. After college, we were roommates in Boston and I was a bridesmaid at his wedding. Brad is more than my friend he is my "brother from another mother."

When I heard about what Brad had done, the only thing that came to mind was -- out of character. I don't know what caused this behavior -- if it was a severe mid-life crisis, survivors' guilt from the helicopter crash that killed his best friend, or something else entirely but never, would I think Brad would find himself in this position. Brad always put family above all else.

Over the last several months, Brad and I have spent a considerable amount of time together in person and on the phone. We have spent countless hours discussing his actions. The one thing I know with every fiber of my being: Brad is so remorseful, so sad, so disheartened and so apologetic for his actions. He knows the damage he has caused to his family and that it is going to be very difficult to make amends, but he is determined. He has told me his sole goal when he gets out of prison is to repair and refocus his family.

Despite what Brad has done recently, please don't judge him solely on this one chapter of his life. Throughout his life, Brad has performed countless, selfless acts of kindness. He has been on the board of trustees at Far Hills Country Day School (where we went to school together), Westminster School and Essex Hunt Club. He received the "Unsung Hero Award" at Far Hills Country Day School for quietly and anonymously donating his time and financial resources to the school.

Brad is the person you call at 2 am, in the pouring rain, to come help change your flat tire. In fact, the home where he is staying on house arrest is the home of a family that he helped.

When Brad heard about their child needing urgent medical care, he dropped everything and flew their child to the best hospital to receive critical care. He did this without knowing the child or the parents previously. Brad was aware of how fortunate he was financially and donated time and resources to many organizations. He frequently used his plane to fly people free of charge to receive medical care.

As a parent, I can also state with confidence and all my heart, Brad is an incredible, highly-involved father to his children – Emily, Eliza, and William. He is involved with their education, sports, and well-being. He has coached his children's ice hockey teams, driven to countless games at Westminster and Colby and encouraged them to be happy, well-rounded people.

Please consider being lenient on Brad's sentence. He has been on house arrest for 9 months and he is sincerely remorseful about his acts. He takes full responsibility for his situation and wants to "complete his time" so he can get out and begin to make amends. He desperately wants to continue to provide for his family as well as rebuild his relationships with them.

Please be kind.

Thank you so much for taking the time to consider this letter of support.

Respectfully,

A handwritten signature in black ink, appearing to read 'Wendy S. Cutler', written in a cursive style.

Wendy S. Cutler

EXHIBIT A3

Mr. William H. Mell III
13 School Street
Hopkinton, MA 01748

March 27, 2019

The Honorable Brian R. Martinotti
United States District Judge for the District of New Jersey
Clarkson S. Fisher Federal Building & US Courthouse
402 East State Street
Trenton, New Jersey 08608

Re: United States of America vs. Steven Bradley Mell

Dear Judge Martinotti,

My name is William H. Mell III and I am the older brother to Bradley Mell, so aside from our parents, I guess you could say I know him well and the longest too. Brad and I along with our other brother Courtney have had a lot of different experiences together ranging from grade school to being members at the same clubs and right on into living together for a period of time in college so I know him very well in a multitude of different situations and environments. Although we went in different directions career wise, naturally our lives are still intertwined to this day. In fact, Brad is the person my wife and I chose to be legal guardian of my 2 girls should anything ever happen to us and to this day that has not changed. We KNOW him and TRUST him!!

Needless to say, we are completely aware of everything and Brad's guilty plea and can **emphatically** tell you this is an aberration from the Brad I've known the past 53 plus years. The Brad I know has always been a hard worker with integrity and determination to not only succeed at a job, but to do it the right way starting when he was 16 years old. At that age he started his own business with a friend washing and detailing the inside/outside of airplanes and cars. He did such a great job that it didn't take long for his business to grow by word of mouth and for the next 4 years or so that was his summer job. He has carried his work ethic and integrity through into the business world where he has made many friends and has earned everyone's respect! In fact, he is so highly respected that he was asked to be on the Board of Directors at a number of places, including the private clubs we belong to as well as private schools and charities.

Another great example of Brad's character is that Brad does Air Lifeline which is now known as Angel Life flights for people that need better medical care in a different part of the country so he either flies them himself to wherever or, at his own expense, has a pilot use his plane to get the patient to wherever they need to go. Brad has also helped raise money for the schools and charities to help others (i.e Brady Camp for underprivileged kids from Newark, Mooreland Foundation, et al.). The list could go on and on but the bottom line is Brad is a man of strong character and integrity!

A third and final example of the Bradley Mell I know is the size of his heart and helping people when they need it. There was a women who started working for W.H. Mell back when our Father first started it and continued working there when Brad took over the business. By this time she was more like family, but when her mother passed away, this women and her family were so devastated that Bradley flew her and her whole family down to South Carolina with his plane and at his expense for 5 days so that they could just be together and hopefully have some good memories come out of the experience. The list could go on but you should get the picture at this point.

As for Brad's family, I want the courts to know that Brad has never been a problem to anyone and he adores his family more than you will EVER know! Brad truly is a good son and has been there for our Mother who is 81 and fighting cancer.

The only concern me or my family have ever had about Brad was the after effects on him when his helicopter went down killing a very close family friend as well as another passenger. This was an accident that completely devastated Brad and frankly, we didn't think the doctor treating him was helping. Thankfully he is now seeing one that most definitely is helping and treating him for PTSD as a result of the accident. I have always been told that prison is supposed to rehabilitate someone from deviant behavior, but in this case, we're not talking about a guy sitting in a room on a computer trying to lure little children or driving around in a van looking for children. We're talking about a guy who suffers from a proven illness (as diagnosed by the Psychiatrists on both sides of the table) brought on by a tragic event and engaged in conduct that was condoned by the family. Heck, after the facts came to light of the accusing family's criminal actions, even the prosecutor acknowledged she felt badly for Brad but has to do her job. The fact that the Prosecutor has now seen more evidence and has come to a plea agreement should also say something. If the Court truly wants to really help him, then do the right thing and put him on probation for 5 or 7 years and make it mandatory that he visit his therapist once or twice a week.

Should you decide to send him to prison, then I ask the Court to be as lenient and as merciful as possible in sentencing Brad and follow the terms of the plea agreement, which is, that Brad will be sentenced to federal prison for 60 months.

Thank you very much for considering my letter.

Respectfully,



William H. Mell III

EXHIBIT A4



SOBEL & CO. LLC

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March 23, 2019

The Honorable Brian R. Martinotti
United States District Judge for the District of New Jersey
Clarkson S. Fisher Federal Building & US Courthouse
402 East State Street
Trenton, New Jersey 08608

Re: United States of America Vs. Steven Bradley Mell

Dear Judge Martinotti,

I am writing on behalf of my friend Steven Bradley Mell (Brad). I am a Certified Public Accountant in the State of New Jersey and have known Brad for over twenty years. We first met when Brad was working with his father in the Municipal Bond Broker/Dealer they owned, WH Mell Associates. WH Mell was a client of our firm where we provided audit and tax compliance services as well as tax services to his family and other business ventures he was involved with.

Over the history of working together with Brad, I have had the privilege of getting to know him beyond the debits and credits of his business. I came to know Brad as both a businessman and as a friend; a friend with a good foundation of values and a strong commitment to his family and his community.

On a professional level, Brad and I discussed many complex business issues that required decisions, some that were crystal clear and others that were not so clear. My overwhelming experience with Brad's decision making was that he chose the course most in line with appropriate regulatory, legal and ethical considerations. Based on these experiences, I believe Brad to be a client that treated me with appropriate professional respect and one that tackled the issues we needed to address with strong and appropriate values.

Through our business relationship, I also got to know Brad on a personal level. Over twenty years you learn a lot about someone. What I learned is that Brad was a dedicated husband, father and friend. Family and friends were the most important things in his life and his business was merely the vehicle to help him provide the very best he could for them. He was incredibly charitable in his giving, both time and money, to the community he lived in and the schools his children attended. Though his kids attended out of state boarding high schools, Brad made every attempt to be there for their games and other important events.

Honorable Brian R. Martinotti

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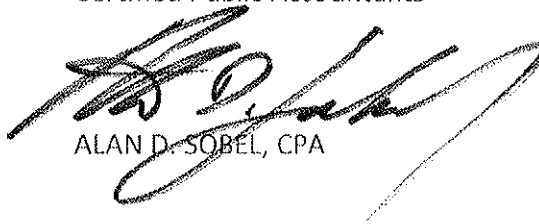
Several years ago, Brad's friends were in a fatal crash on a helicopter that Brad owned. He was also supposed to be on that flight, but another business commitment prevented him from going. You can only imagine the personal devastation, loss and guilt that Brad experienced. Yet, even though he faced potential liability as the owner of the aircraft, Brad was there for his friends' families and ensured that they received as much financial consideration and personal support that was possible. It was the kind of person he was, and a testament to his strong character, that he did not hide from those responsibilities or run the other way.

As you deliberate on the sentence for Mr. Mell, I respectfully request you consider my personal experiences with Brad. It is my hope that you will weigh his guilty plea against the balance of his contributions to his family, community and friends and recognize all that he offered so far during his lifetime. He still has much to offer and I ask that you follow the terms of the plea agreement when you determine his sentence so he can return to society and make more positive contributions.

Thank you very much for your consideration of my letter.

Very truly yours,

SOBEL & CO, LLC
Certified Public Accountants



ALAN D. SOBEL, CPA

EXHIBIT A5

May 24, 2018

To Whom it may concern –

I recently heard that Brad Mell is being held in jail. I have not spoken to Brad in several months and have no details of what he has been accused of doing. I am writing because I have known Brad for most of my life since we were childhood friends, and I wanted to share my observations of and personal interactions with Brad in support of his character.

Brad and I grew up on the same street. While we were not extremely close friends, we attended the same school and played on the same sports teams. Brad has always been a person who stood up for people and treated everyone with respect regardless of their age and role. He was well-liked by everyone and would always stick up for the underdog and be a trusted friend. In middle school, Brad would occasionally invite me to his house let me ride on his mini-bike or snowmobile that he had worked very hard to buy for himself. Starting in grade school, Brad was very driven and always had a job of sorts, and yet was very generous with his coveted “toys” and loved to share the excitement of riding them.

Brad and I would see each other occasionally during our high school and college years. As adults, we both eventually moved back to the community in which we grew up in New Jersey to raise our families. In my observation, Brad has been a very involved father, dedicating time to his kids at their school and in sports.

I can't remember all the history, but Brad began to work in his Dad's business (which I recall as good, but in somewhat of a decline before Brad became more involved). Similar to his childhood, Brad worked extremely hard, strengthened the business and built significant professional success. Brad has applied this success to help others. He has been an active volunteer and board member for many local organizations. He gives generously of his time and money to help causes, organizations and people.

The most important thing I remember about Brad, and why I am writing this letter, is what Brad did for me when my father died. Brad knew my father and knew how close we were. Brad had a similar relationship with his father. My father died in July 2012 and we planned the funeral shortly thereafter. It was in the middle of summer and many of my friends were on vacation and busy with their own young families. Entirely on his own, Brad arranged to fly about 8 of my closest friends to attend the service and return them back to their own families the same day so as not to disrupt family vacations. While Brad knows these folks through mutual friends, these were my closest college friends – not his. He knew how much the effort would mean to me at a very difficult time. He didn't ask anyone to help pay and wasn't doing it for any kind of personal gain, but rather because he knew how great it would be for me to have such a strong support network.

These are my personal experiences with Brad and I have always known him to have a great character.

Sincerely,

Perry Hall

EXHIBIT A6

LtCol & Mrs. Hunter R. Kellogg USMC (Ret.)
111 Hart St. Beverly, MA. 01915

3 April 2019

The Honorable Brian R. Martinotti
 United States District Judge for the District of New Jersey
 Clarkson S. Fisher Federal Building & US Courthouse
 402 East State Street
 Trenton, New Jersey 08608

RE: United States of America Vs. Steven Bradley Mell

Dear Judge Martinotti,

I write today in regards to the case referenced above; my wife and I know Mr. Mell by "Brad", and will refer to him as such herein. We are aware of Brad's wrongdoings, and acknowledge their seriousness, but also his acceptance of his responsibility. He has offered us no excuses, only context. We write to you in hopes of conveying a wider picture of who Brad Mell is as a person, not to excuse his past transgressions.

My wife and I met Brad Mell eight years ago, as his eldest daughter, Emily, was dating our son; they were both students at Westminster School. Brad was serving as a beloved member on the Board of Trustees at Westminster and we met him when he came in for a trustees meeting. We knew Emily long before we met her father, and I cannot more deeply convey the level of *her* character, and that of Eliza and William, her two siblings, which Courtney and I feel is a direct testament to how Brad and Kim raised their children.

The Mell family had means beyond ours; serving in the military is less lucrative than the private sector. What struck us about Emily was how responsible she was, how caring she was and how, after meeting her, one would never know she came from a family with such means. Her sister Eliza was also at Westminster, and Courtney and I were impressed by how sincere she was as well. I emphasize this because at a small private school, one can often find students who flaunt their families' wealth, students who are entitled and elitist, and students who, without flinching, will tell you to which family they belong. Our son was the only active-duty student at the time and suffered no fools. He and Emily hit it off immediately, and went on to date for six years.

When Courtney and I see our children's peers, it's usually clear how they were raised. With pride, I submit that our children are respectful, socially active, global, no-nonsense children - and we are honored to see the adults they have become. Brad's children are likewise, demonstrating every characteristic you'd want in your own child: inquisitive, helpful, down-to-earth, caring, bright, thoughtful and responsible.

Incredible children don't "happen", they are the result of thoughtful guidance, gentle nudging, deep support and truly loving parenting. One out of three stellar children could be a fluke; three of three children being the kind of person you'd want your children to befriend, means their parents did it right. Brad was the father who answered every call, spent time with his children, and tried to attend every game his children played in, no matter life's demands; he is that devoted to his children, and they are the better for it.

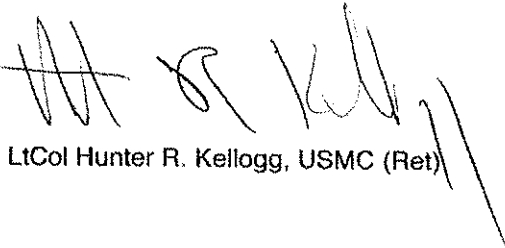
Neither Courtney nor I condone what Brad has done; and in our many conversations with him, it is crystal clear he knows that he needs to right his wrong. He made a mistake and should be held responsible for *his* role. Months ago, Courtney and I were concerned about Brad's emotional well-being; the despair was clearly evident when we spoke to him. Today he seems at peace, knowing that there is an end in sight and he will pay his debt to society, as any offender should.

LtCol & Mrs. Hunter R. Kellogg USMC (Ret.)
111 Hart St. Beverly, MA. 01915

One could learn of his story and cast judgment immediately - this is a scandalous story - the media would cover it with fanfare. But there are always two sides to every story, and a one-sided telling would prevent anyone from knowing who Brad *really* is. We are all human, and we are all flawed, just to varying degrees. Brad is a man who made a mistake. He's not a predator. He's not a monster. He acted without thinking about the larger picture. I plead with you Judge Martinotti to recognize that Brad made a mistake, and I ask for your level review, judgment and (hopefully) lenient sentencing. He has paid dearly for his mistake: he lost his family's business, his wife, the structure of his nuclear family and countless friends. I humbly ask that you consider following the terms of Brad's plea agreement, and sentence him to no more than 60 months.

I have never written a letter such as this, and I agreed without hesitation when asked. Brad is a man who is so much more than his mistake. I ask for your mercy Judge Martinotti, and I thank you for taking the time to read my letter.

Most respectfully,

A handwritten signature in black ink, appearing to read 'H. R. Kellogg', followed by a long, sweeping horizontal line that extends to the right.

LtCol Hunter R. Kellogg, USMC (Ret)

EXHIBIT A7

Joy Frelinghuysen
P.O. Box 2174
Middleburg, VA 20118

March 15, 2019

The Honorable Brian R. Martinotti
United States District Judge for the District of New Jersey
Clarkson S. Fisher Federal Building & U.S. Courthouse
402 East State Street
Trenton, New Jersey 08608

Re: UNITED STATES OF AMERICA VS. STEVEN BRADLEY MELL

Dear Judge Martinotti,

My name is Joy Frelinghuysen. I have been close friends with Brad Mell since we were children. Our families are friends and we saw each other often at family parties and through the Somerset Hills Country Club in Bernardsville, New Jersey and the Essex Hunt Club in Peapack, NJ.

I am aware of Brad's guilty plea and the gravity of his wrong actions in this case. With that in mind, I feel it is important to tell you that Brad Mell is the last person I ever thought I would be writing this type of letter for. The reason I say that is because what distinguished Brad from the rest of our peers was his sincere concern for everyone around him. While most of us were busy being self-involved teens, Brad always, always put the safety, happiness and well-being of his friends and family before himself.

As we moved from grade school into high school, Brad was the one among us who would be insisting on driving the kid home who may have had too many beers. If someone got broken up with, Brad was the one who would counsel them and help them through it. He has always hated to see anyone in pain. He has always wanted to lift people up and make them feel they can succeed. Brad has a big heart and "wears it on his sleeve." That's what we've always said about him. He is a person of few words but his big, brown eyes reflect his gentle, deep-feeling nature. He is always quietly there for you, never judging but rather actively listening and giving advice only when asked for it.

These loving, supportive aspects of Brad's character found their true north when he had children. In fact, his most beautiful quality is his deep love for his children. Do I realize the irony of this statement given the circumstances of his case? I absolutely do. For all of us who know and love Brad, his poor judgment and choices in this matter will never make sense, but here is what I can tell you for sure. His three children do not have one doubt in their minds about his undying love for them. He is their rock and their source of unconditional love. Ever since this deeply regrettable situation came to light, Brad's kids, Emily, Eliza and William, have been unwavering in their support of him. I think this speaks volumes about his true character. The bond between father and children, through this challenging time, has been something remarkable to witness. I'm not sure I have ever seen anything like it. Brad has never once broken down in front of his children because he is doing everything in his power to minimize

the pain his actions have already caused them. I have spoken to him constantly in the last year and I can tell you that his regret over the torment he has caused his wife and children and the other parties has almost broken him several times. The one thing that has kept him going forward is his commitment to his children. He has tried to assure them that he is, and always will be, there for them and I believe he has succeeded.

Judge Martinotti, Brad is deeply remorseful and ready to serve his time for said actions with a complete understanding of why that needs to happen. I beseech you to follow the terms of the plea agreement and keep his time in federal prison to the 60 months he has been sentenced to. Some people come out of experiences like this with a renewed commitment and zeal to being the best person, the best father, the best son, the best friend and the best citizen they can be. I am utterly certain that Brad will be one of those people.

Thank you so much for taking the time to read this letter.

Sincerely and respectfully,



Joy Frelinghuysen

EXHIBIT B1

Catherine M. Barber, Ph.D.

Clinical & Forensic Psychology

30 Washington Avenue, Suite B-1

Haddonfield, New Jersey 08033

Phone: (856) 427-6766

Fax: (866) 315-8220

NJ License No. 3393

PA License No. PS-006085-L

May 31, 2018

The Honorable Cathy L. Waldor
U.S. Magistrate Judge for the District of New Jersey
Martin Luther King Building & U.S. Courthouse
50 Walnut Street
Newark, New Jersey 07101

Re: Steven Bradley Mell
Mag. No. 18-7094 (CLW)
Confidential report of psychological evaluation

Your Honor:

In response to the Court's recent Order, I have conducted a psychological evaluation and risk assessment of Mr. Steven Mell, who is presently detained at the Essex County Correctional Facility, facing three counts of Online Enticement of a Minor and one count of Travel with Intent to Engage in Illicit Sexual Conduct. In light of the serious nature of the allegations, an evaluation of Mr. Mell's risk of potential harm to himself or others was ordered, to weigh along with other factors in the Court's consideration of whether to grant Mr. Mell pretrial release on bail.

I conducted the evaluation of Mr. Mell on 5/22/18 at the Federal Courthouse in Newark, in the U.S. Marshals' holding area, as arranged through the U.S. Pretrial Services Agency. I conducted a psychosocial interview and mental status examination, supplemented by completion of several structured professional judgment tools for the assessment of risk of harm to self or others. I also reviewed and considered documents provided by the U.S. Pretrial Services Agency, including the Criminal Complaint with attachments; the Pretrial Services Report; and a copy of a New Jersey firearms purchase permit application Mr. Mell completed the day prior to his arrest.

I discussed the purpose and scope of the evaluation with Mr. Mell, and informed him that a report of my findings would be furnished to the Court via the U.S. Pretrial Services Agency. He understood that the evaluation was Court-ordered, acknowledged the consequent absence of confidentiality, and readily agreed to cooperate. His demeanor was respectful and open. He appeared to be doing his best to give accurate and thoughtful responses to all inquiries.

Mental status examination findings

Steven Bradley Mell (date of birth 12/15/65) is currently 52-years old. He presented for the evaluation neatly dressed in his jail-issued uniform and demonstrating good attention to his personal grooming and hygiene. He was in no immediate apparent physical or mental distress. He remained fully alert and well oriented throughout the examination and interview. He made good eye contact and his demeanor was calm, open, talkative, and cooperative. No unusual motor behaviors suggesting neuropsychological impairment were noted. Speech was normal in rate, rhythm, volume, tone and articulation. Attention, concentration, and memory (both short- and long-term) all were unimpaired. His thought content was reality-based and did not reflect paranoia or any other delusional ideas. No perceptual abnormalities such as auditory or visual hallucinations were reported or inferred from Mr. Mell's observable behavior. His thought processes and verbal expression were well organized and rational, and reflected excellent abstract reasoning capacity. Current judgment and impulse control appeared intact. General intellectual functioning appeared to be within the high average range. Mr. Mell reported no history of smoking cigarettes or using any illicit drugs, and stated he drank very little alcohol (one after-dinner drink approximately twice per month) prior to his incarceration.

Mr. Mell's affect (outward expression of inner mood states) ranged fully and was consistent with subjects under discussion as they arose during the interview. He appeared to have no current difficulties managing frustration or anger, and did not express or imply any hostility toward others, either generally or specifically. He was mildly tearful at a few points in the interview, but was able to readily regain his composure. He stated he did not consider himself depressed, either previously in his life, or situationally due to his current confinement and facing serious criminal charges. He expressed feeling that he has substantial support from family and friends, which helps in this regard (see also *Current adjustment* below). He has a history of treatment for anxiety following a significant life trauma (discussed further below), but he has never regarded this anxiety as a mental health problem *per se*.

Overall, the mental status examination yielded non-significant findings. There is mild to moderate situational anxiety noted, as well as mild to moderate sad mood, both related to current separation from family, missing family events, and concerns regarding the ongoing operations of his business during his absence. Symptoms are not sufficient in number or intensity to warrant diagnosis of a mood or anxiety disorder at this time. There are no signs of disturbance of the thought process, intellect is intact, and there are no indications of impaired self-control. Thus, on the basis of the mental status examination, there are no concerns raised about any condition(s) which might foreseeably impair Mr. Mell's judgment or impulse control relevant to risk.

Relevant background

Background was obtained directly from Mr. Mell, without benefit of collateral information other than review of the Pretrial Services report. Mr. Mell gave every indication of being straightforward in the interview. He related he grew up in Far Hills, New Jersey. His parents were divorced. He stated he was diagnosed with dyslexia as a child, but did not require special education services and “worked around it.” As a teenager he served as a volunteer EMT. He attended Boston University, and from the time he was a college junior, for approximately four or five years (1987-1992), he served as an auxiliary police officer in Brookline, Massachusetts. He reported he has been married to his wife, Kim, for 27 years. They have three children, two young adult daughters and a son in high school. Mr. Mell has worked in the financial industry for many years and achieved a high degree of success. He began working for his father and eventually took over leadership of the business.

Mr. Mell suffers from primary sclerosing cholangitis (bile duct disease), a serious and progressive liver disease with which he was diagnosed 17 years ago. His condition may eventually require a liver transplant. He receives care regularly for this condition in New York and at the Mayo Clinic. He also suffers from Crohn’s disease, a chronic inflammatory bowel disease. He appears to be coping well with these chronic and serious health problems.

No other significant health concerns were noted, including any that could foreseeably impact Mr. Mell’s psychological functioning. He reported he has lost nearly 40 pounds over the past two years, but denied this was associated with depression or any mental status changes. He noted he had a long-standing habit of drinking coca-cola, up to a case per day, for many years, but in the past two years reduced this to approximately two six-packs per day. He suffered caffeine withdrawal headaches temporarily but they have resolved.

Mr. Mell, a licensed airplane and helicopter pilot, experienced a major psychological trauma when his best friend, Will, died in a flight accident in 2008, aboard a helicopter owned by Mr. Mell’s company. He explained that he would have been a member of the excursion himself, but was prevented from going on the trip (a golf outing with friends) at the last moment due to work constraints. He gave up his spot to another friend, who was seated in the front of the helicopter alongside Will at the time of the crash. Mr. Mell believes that if he had been on the trip, he might have been able to assist Will with whatever situation arose that led to the crash.

In the aftermath of this loss, Mr. Mell was emotionally devastated. In 2010, he experienced his first panic attack, which he attributed to long-term effects of the shock trauma over the accident. His family physician prescribed Paxil (commonly used to treat both anxiety and depression), and Mr. Mell consulted a psychotherapist as well. At times his doctor also prescribed Xanax and Klonopin, both shorter-acting anxiolytic medications. He stated he has taken Xanax “as needed” over the past few years. Mr. Mell stated he had never had any prior mental health conditions, and feels in recent years his reaction to this traumatic loss has substantially resolved. He stated, “I’m at peace with everything. I’m still good friends with with

Will's wife and family." In fact, Will's mother has volunteered to serve as his third-party guardian if he is granted bail.

Current adjustment

Mr. Mell described his arrest by the FBI as temporarily traumatizing; he stated, "they came and arrested me with an overwhelming show. It was a blur. There were five or six laser targets on me." He stated he experienced a "huge panic attack" at the time of his arrest, but this has not recurred. Upon his entry to the jail, he was initially placed in the regular population, but somehow word got out to other inmates about the nature of his charges, and his cellmate threatened him. Consequently he was placed in "isolation" for six or seven days. He stated, "I thought that would be extraordinarily stressful, but actually I coped with it okay." He related he was able to converse with the inmate in the cell adjoining his, and he found this "comforting."

At the time of this interview, Mr. Mell stated, "I'm doing pretty well." He has been returned to an open cell block and stated he gets along well with his current cellmate. He is receiving medication (he could not recall the name) to help him cope with withdrawing from Xanax. He stated he has had no significant crying spells or panic attacks while in the jail. He added, "To this day, I don't feel depressed; I don't feel angry. I do miss my family."

With regard to his severely stressed marriage, Mr. Mell acknowledged, "I think it's done; I've got to give my wife her space." But he verbalized determination to work cooperatively and amicably with his wife, commenting, "My parents had an awful divorce, and I don't want that." He is clearly saddened by the impending loss of his marriage due to his actions, stating, "We were married for 27 years. We had marriage counseling at one point, then we drifted apart, but we were never separated before. With the business I'm in, it (the opportunity for an extramarital affair) could have happened 1,000 times, but never did. Our life was actually in a good place prior to this." He appeared overall to have a realistic appraisal of the situation. He stated, "I understand that my wife wants a divorce. I still love and care about her very much. I'd like to reconcile, but that's not going to happen."

In terms of coping not only with his criminal prosecution but with the turmoil his family is now experiencing, Mr. Mell related, "Our friends have been phenomenal, to both Kim and me. I'm very close to my three kids; they have been 'rock stars.' I missed my daughter's graduation from Bucknell on Sunday, and that hurt." He indicated he has asked his family "not to visit me in jail. I don't want them to see me here. I'm hoping for bail, but if I don't get bail they will be added to my visiting list." Mr. Mell stated that everyone important to him in his life was aware of his arrest and his charges, and he is receiving "a lot of support." His mother is posting his bond, and his two brothers are helping. He added that a number of people have already offered to serve as character witnesses for him. And, as noted above, the mother of his friend Will has offered to serve as his third party guardian, if needed, should he be granted pretrial release.

Risk assessment findings

Self-harm and suicide risk were evaluated using the Columbia-Suicide Severity Rating Scale (CSSRS) and the Suicide Assessment Five-step Evaluation & Triage (SAFE-T) procedure, measures endorsed by the U.S. Department of Health and Human Services' Substance Abuse and Mental Health Services Administration. **My opinion, to a reasonable degree of psychological certainty, is that Mr. Mell currently and foreseeably presents with a low risk of suicidal or self-harming behavior, based on the following findings.**

Mr. Mell reported he has had passive thoughts about dying in the past, in the aftermath of the helicopter accident that claimed the life of his friend, and in which he "should have been" a passenger himself, but for a work requirement that prevented him attending the outing. He stated it has been a long time since he these type of thoughts, and he asserted he has never wished to be dead, or formulated plans or seriously contemplated injuring himself or taking his own life. There is recent family turmoil due to his conduct that eventuated in the current charges, but he appears to be handling this well and verbalizes a strong commitment to working through divorce proceedings with his wife in a civil and supportive manner. He stated, "I live for my family. I will be here to my very last breath to support them, including my wife." He is managing the current stress of incarceration and his legal situation well.

Mr. Mell has also demonstrated a long-standing commitment to his own health and well-being through his active engagement in treatment for his serious liver condition. As noted above, there are no current signs or symptoms of a mental disorder that might foreseeably distort Mr. Mell's thought process or judgment, or increase his impulsivity, in a manner that might increase his risk of self-harm. He does not currently exhibit or express loss of interest or pleasure in life; hopelessness; anxiety/panic; insomnia; or command hallucinations. There is no recent change in treatment.

A salient potential risk factor in this case was thoroughly addressed, that being Mr. Mell's failure to report, when asked during his Pretrial interview about access to weapons, that he had applied for a firearms permit the day prior to his arrest. He explained he was having a problem on his property, which encompasses 30 acres and adjoins an additional 1000 acres of open land, with coyotes appearing near the home and posing a possible threat to the family's puppy. He discussed this with several friends, who encouraged him to purchase a shotgun to address it. He stated he applied for a permit to purchase two handguns at the advice of the clerk who provided him with the permit, who reportedly said this would prevent him from having to re-apply if he decided to purchase a handgun in the future (two being the limit for one permit); however, Mr. Mell stated he only intended to purchase a shotgun. He also stated that one of the friends who advised him about this problem had text messages in his phone that would fully corroborate his account. Further, he stated he had "no idea" he was to be arrested the following day. In fact, he "thought just the opposite. The police were aware since December about this, but it had gone quiet. Nothing had happened except for the family taking civil action." He added, "The last thing I would do if I had any interest in self-harm would be to go to Bedminster and get a permit."

With regard to the section of the application inquiring about mental health issues, Mr. Mell said he never considered his anxiety following the helicopter accident as a mental health problem, *per se*. His general practitioner prescribed medicine for him, which he utilized for a few years but had only taken as needed more recently, and he considered his consultation with a therapist following the accident to be a healthy thing to do to help himself cope with the loss of his friend and come to terms with his sense of personal accountability. He stated, "I didn't get it, that my anxiety was a mental health issue."

Although Mr. Mell may have been minimizing to himself the nature of the psychological problems he had, and certainly should have realized when directly asked about firearms during his Pretrial interview that he should disclose the fact he had just submitted an application for a purchase permit the previous day, I was satisfied after this lengthy conversation that his steps to obtain a firearm permit do not represent actions taken in pursuit of a means of either ending his own life or harming anyone else.

Proceeding to the issue of his potential risk of harm to others, this was evaluated based on the mental status examination and behavioral observations, supported by further reference to two structured professional judgment tools, the HCR-20, a general violence risk measure, and the Static-99R, which assesses risk factors specifically associated with sexual violence.

The HCR-20 facilitates the systematic assessment of 20 historical, clinical, and risk management factors empirically shown to be associated with violence, defined as "actual, attempted, or threatened infliction of bodily harm of another person...The focus is on behaviors that cause or have the potential to cause physical or serious psychological harm. Serious psychological harm includes fear of physical injury, as well as other emotional, mental, or cognitive consequences that substantially interfere with health or well-being" (HCR-20 v.3 professional manual, pp. 2-3). None of the 20 factors of the HCR-20 were rated as present. No prior record of violent acts, including domestic violence, was detailed in the Pretrial Services Report or disclosed by Mr. Mell. He reported his wife "did call the police" due to a disagreement "within the past couple of months," but no charges came of it. He reported he was charged with one DWI offense while in college; he was held in jail for a few hours, then released, and as a consequence of the offense he lost his driver's license for several months. On the Static-99R, where scores range from -3 to 12, Mr. Mell's score of 0 places him in the "below average risk" range. Among a sample of routine sex offenders (non-high risk), between 2.2 and 3.5 out of 100 offenders with a score of 0 would be expected to be charged or convicted of a new sexual offense by five years post-release.

Mr. Mell appears highly motivated to succeed and to be fully compliant with all requirements imposed by the court if granted pretrial release. He stated, "I have a family, including my wife; a firm, and a life to live. I want to see our grandkids. I am not going anywhere; I've got to work. There is not a chance I will attempt to contact the victim. I need to get back to work to support my family. I don't want to jeopardize that if I am granted bail."

Integrating all of the above findings with the mental status assessment, behavioral observations and my professional judgment, in conjunction with review of the documents provided **it is my opinion, to a reasonable degree of psychological certainty, that Mr. Mell currently and foreseeably presents a low risk of harm to himself or others** if released on bail pending resolution of his charges. This opinion refers to his foreseeable risk of suicidal or self-injurious conduct, and of general harm to others, as well as any risk of harm similar in nature to the conduct with which he is presently charged.

Respectfully submitted,

A handwritten signature in black ink, appearing to read "Catherine M. Barber". The signature is fluid and cursive, with a long horizontal stroke at the end.

Catherine M. Barber, Ph.D.

EXHIBIT B2

RICHARD B. KRUEGER, M.D.

Diplomate in Psychiatry, American Board of Psychiatry & Neurology **210 EAST 68th STREET, #1-H**
Diplomate, American Board of Internal Medicine **NEW YORK, NEW YORK 10065-6024**
Diplomate, ABP&N, Added Qualifications in Forensic Psychiatry **TELEPHONE (212) 517-6624**
Diplomate, ABP&N, Added Qualifications in Addiction Psychiatry **FAX (212) 517-4073**
e-mail **rbk1@cumc.columbia.edu**

September 19th, 2018

Robert A. Bianchi, Esq.
Attorney at Law
The Bianchi Law Group, LLC
4 York Avenue, 2nd Floor
West Caldwell, New Jersey 07006

By Federal Express; telephone is: 862-210-8570

Re: Psychiatric and Risk Assessment of Mr. Steven Mell

Dear Mr. Bianchi:

I have evaluated Mr. Mell, with my report below:

Identifying Data & Chief Complaint: Mr. Mell is a 52-year-old white male who is a U.S. citizen. He was raised in the Catholic religion and attended church twice a month before he was placed under supervision by Federal Probation. He speaks only English. He is married x 1, but has been separated from his wife, who is 52, for 2 months and is in the process of a divorce. He has three children, a daughter, 22 a daughter, 20 (both of whom are away at college), and a son, 15, who is living with his mother in their eight-bedroom house in Far Hills, New Jersey, where they have lived for nine years. Mr. Mell two months ago following his arrest for sexual abuse of a minor moved to a one-bedroom apartment located over a garage in Far Hills. Mr. Mell's chief complaint was, "I am seeing you for an evaluation."

Sources of Information: Included the following interviews:

1. I interviewed Mr. Mell for 4 hours on August 29th, 2018 and a follow-up telephone call on September 18th, 2018

It also included a review of the following materials, provided by your office:

1. The U.S. Complaint
2. The Search Warrant Inventory

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3. The Hunterdon County Complaint
4. The Hunterdon County Discovery, 1-496
5. Memorandum in Support of Release of Exhibits
6. Lilly Cannon-2 videos-recorded statements
7. Laura Cannon recorded statement
8. Anthony Cannon recorded statement
9. Recorded statements of Kajal Mehta
10. Recorded Statement of Sarah Smith
11. Forensic Interview of Lilly Cannon-parts 1 and 2
12. Statement summary of Lilly Cannon 's Statement of December 21st, 2017
13. Statement summary of Laura Cannon's statement of December 21st, 2017

Informed Consent for the Examination: Prior to the examination I informed Mr. Mell that I was performing an evaluation at the request of his defense attorney and that whatever he said to me could be included in a report to him or to her, and that his defense attorney may, in turn, choose to present this to the Court and the prosecution. I also presented information on the various psychological and written tests that I would be administering. Mr. Mell understood this information and signed appropriate consent forms.

History of the Present Illness: Mr. Mell said that he was born in Morristown, New Jersey. He said that, as far as he knew, his mother's pregnancy with him and delivery of him were normal. He was raised in Far Hills, New Jersey. Asked what growing up was like, he said that there was a very traumatic divorce when he was 12 or 13. When asked about his educational history, he said, "I went to St. John's in the Mountains school in Bernardsville, New Jersey." "I then went to Far Hills Country Day School, which is a private day school in Far Hills, for kindergarten through 9th grade." Mr. Mell said that when he was in the 6th grade, he was thought to have severe dyslexia. Some of his teachers suggested that he was mentally retarded or severely impaired. He said that two teachers took him on and helped tutor him. He said that he had extensive diagnostic testing including an EEG and brain imaging and ultimately was diagnosed as having severe dyslexia by a Ms. Nelsnisk, an expert in dyslexia at the time. She worked with him for three years, seeing him twice per week when he was in the

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7th, 8th and 9th grades. Mr. Mell with this was able to graduate from 9th grade in the Far Hills Country Day School in 1981.

Mr. Mell then went to Tilton Preparatory School in Tilton, New Hampshire, for three years, graduating in 1984 at the age of 18. He graduated somewhat late because his parents had held him back for a year. Mr. Mell said that he had gone to schools that were extremely competitive and that most of his classmates had gone to Harvard, Yale, or Princeton. He felt inferior to them and said that he was made fun of periodically. However, he was exceedingly achievement oriented. He said that he became a pilot at the age of 16. Throughout Far Hills Country Day School and Tilton he was an avid hockey and lacrosse player. He said that he followed on the steps of his father who was a champion hockey player at Yale who still held the record for most goals scored in a single game. Mr. Mell also rode horses. Additionally, Mr. Mell became certified and worked as an emergency medical technician in Far Hills and in Boston from 1981 to 1988.

Mr. Mell then went to Boston University (BU). He said that he initially applied and was not admitted, but then went to night school at BU and received good grades and was eventually admitted to BU. He was a student there from 1984 to 1988, graduating in 1988 with honors. He said that he continued to work as an EMT during his time at BU. He said that he received a stork pin for delivering a child. He said that he received a B. A. Degree with a major in Psychology and a minor in Business. He also worked as a voluntary police officer in Brookline, working at this 15 to 20 hours a week from 1986 to 1990.

Mr. Mell said that he then decided to become a government bond trader and worked for State Street Bank in Boston from 1988 to 1992. He moved to New Jersey in June of 1992 after he married his wife. He was 27 and she was 28 when they married. They had dated and had had a sexual relationship for seven years prior to their marriage. He said that he married her because he loved her and said, "I still do." He said that his father had wanted him to come into his father's business right after he graduated from BU, but Mr. Mell had wanted to be on his own and worked for State Street Bank.

Mr. Mell eventually began working in his father's firm in 1992 and moved with his wife at this time to Mendham, New Jersey, where they lived in a small rental apartment. They lived there for several years and ultimately built a house in Bedminster, New Jersey, and moved there in 2006. Mr. Mell continued to live there until he was arrested. Mr. Mell eventually took over his father's firm, a municipal bond trading agency called MH Associates, becoming President and CEO of that company. Mr. Mell said that there were only 4 employees when he started working for MH Associates, but by the time that Mr. Mell left the firm, there were 20 employees. Mr. Mell said that he also started his own financial services firm called Goldstein Capital Management. He said that he had approximately a billion dollars under management in his father's firm and the Goldstein Capital Management firm. He said that his businesses had done well.

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Asked how he got into trouble, Mr. Mell said that on October 9th, 2011, there was a helicopter crash. His company owned a helicopter and he had been scheduled to fly with his partners and friends to a meeting. He could not make it so he did not accompany his associates in the helicopter. His associates on the return helicopter trip crashed with Mr. Mell's friend being killed and his partner being severely injured. Mr. Mell said that this came as a great shock to him and that he thought that he should have been in the helicopter. He felt extremely guilty at being alive and continued with and could not dispel the feeling of survivor guilt. He said that following this crash he began psychotherapy with Dr. Adam Price, a psychologist, and continued in this on a weekly basis until 2017 when he said that Dr. Price's son committed suicide and Mr. Mell thought that this detracted from the quality of the therapy that he had been receiving and he stopped seeing Dr. Price. Mr. Mell following the crash also saw his local medical doctor who started him on Xanax and Paxil for anxiety and depression. He said that he had continued taking these medications since. He said that he found his medical doctor extremely supportive and had continued to meet with him weekly in supportive sessions. Mr. Mell said that this crash also resulted in litigation that extended over several years with eventually a payout of \$25 million dollars to his partner and to his friend's family.

Mr. Mell, asked further how he got into trouble, said that on May 8th, 2018, he was arrested by Federal authorities for 3 counts of Online Enticement of a Minor and 1 count of Travel with Intent to Engage in Illicit Sexual Conduct. Mr. Mell said that at 6:00 AM in the morning he awoke to the presence of some 60 FBI agents with multiple red laser targets on his chest when he opened the window to see what the commotion was outside of his house. Mr. Mell said that he had applied for gun permit for a long rifle to shoot foxes on his property the day before the FBI came, which he thought explained the great force that was involved in this second arrest. He said that subsequently, when he was in federal custody, he was additionally charged by the Hunterdon County with sexual assault, specifically by engaging in sexual penetration with a J. Doe when J. Doe was under the age of 16 and he was greater than 4 years older, a crime of the second degree. This charge alleged that he had begun communicating electronically with J. Doe and that he had had sexual intercourse with her.

Mr. Mell was incarcerated for approximately three weeks, initially in Newark and then in a local jail. Bail was initially denied, but following an evaluation it was determined that he was not a risk to himself or others. He was released on one million dollars bail. Mr. Mell said that he had a Federal Pretrial Services Officer with whom he spoke with two times per month.

Past Mental Health History: Mr. Mell in the 6th grade had an extensive evaluation including an EEG, brain imaging, and psychological testing and was found to have dyslexia and was treated by Ms. Nelsnisk. He said that testing at that time also found him to be very bright with an IQ of 130. Mr. Mell said that he had been involved with his wife from 2008 onwards in periodic couples therapy and that they had also seen a couple's sexual therapist. He said that he and his wife had had a good sexual life initially, but that things had fallen off over the years. He said that he last had sexual relations with his wife in March of 2018. He also said that he had seen Dr. Price, a psychologist, in psychotherapy from 2011 to 2017. Mr. Mell said that

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recently he had been seeing Ms. Dorothy O'Keefe, a social worker in Chester, New Jersey, once per week and a psychologist named Dr. Daniel Water in supportive psychotherapy. Asked about any thoughts of or attempts at suicide, Mr. Mell denied prior suicide attempts and current suicidal intent or plans.

Past Legal History: Mr. Mell had one arrest for driving under the influence in Boston when he was in college at the age of 18. He had to attend classes for three months. He subsequently was arrested for the index charges of sexual assault by Hunterdon authorities and for online enticement of a minor and traveling with the intent to engage in illicit sexual conduct by federal authorities.

Substance Abuse History: Mr. Mell said that he began using alcohol in college. He said that it was never a problem for him and that his arrest for DUI was the only such arrest that he had had. He denied to detailed questioning the use or abuse of alcohol. He said that he had smoked cocaine perhaps 10 or 15 times in the distant past. He denied any use or abuse of other drugs including marijuana, hallucinogens, and other substances. He said that his wife had never complained about his alcohol or drug use.

Head Injury History: Mr. Mell said that during his sophomore in college he was hit on the head and knocked cold when he tried to swing under a door and the header of the door that he was trying to swing from broke and he struck his head. He was seen at Morristown Hospital and released; he said that there had been no sequelae from this fall.

Past Medical History: Mr. Mell is 6 feet 1 inch tall and weighs 180 pounds. His weight had been as high as 220 pounds, but he said that the weight of 180 pounds had been stable for years.

Mr. Mell said that 15 years ago he developed weight loss, bleeding, and abdominal pain. He saw a number of doctors, but eventually was seen by Dr. Henry Bodenheimer, a hepatologist at Beth Israel Hospital in New York City. Dr. Bodenheimer diagnosed him with primary sclerosing cholangitis, or PSC. (PSC basically results in a gradual atrophy and dysfunction of the biliary collecting system in the liver such that at some point the liver is unable to eliminate and release toxins and a liver transplant is required). Mr. Mell had this diagnosis confirmed at the Mayo Clinic and regularly sent laboratory results to the Mayo Clinic for monitoring. Mr. Mell said that he had been treated for 15 years for this disease with a compound called Urso Forte, at a dose of 500 mg three times per day. PSC is fatal disease with an average life expectancy after diagnosis of 8 to 10 years. Mr. Mell said that Dr. Bodenheimer told him that he was in the 5% of patients who survived beyond this. Mr. Mell said that he last saw Dr. Bodenheimer in the fall of 2017 where all of his labs were normal.

Mr. Mell also said that 15 years ago he was diagnosed with Crohn's disease (which is associated with a significantly increased risk of colon cancer). He said that he had had monitoring colonoscopy on a yearly basis and that no evidence of cancer had been found. He

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had had no serious attacks of abdominal pain or other symptoms and had not required surgery. Mr. Mell said that he had general practitioner, Dr. Arthur Scott Campbell, in Bedminster, New Jersey, who had taken care of him for 15 years. He spoke very highly of Dr. Campbell, whom he saw on a weekly basis. Mr. Mell had not had any surgeries. He thought that he might be allergic to penicillin.

Past History/Family History/Social History: Mr. Mell's father died at the age of 71 in 2008. "He killed himself." Mr. Mell said that his father was an alcoholic. "He got sober three years before his death, but became very depressed." "My father never knew how to love." "He was very athletic, setting the record for most goals scored by a hockey player in one game at Yale." Mr. Mell became very tearful as he discussed his father. Mr. Mell's mother is 82 and lives in South Carolina. "She is great." Mr. Mell has a brother, 56, who lives in Boston. "He is lazy." "He has never worked and is a stay-at-home father." Mr. Mell has another brother, 53, who is a pilot and lives in Palm Beach and is single "and always will be." Mr. Mell's wife, Kimberly, is 52. "She is really a good person." "I still love her." "I am devastated by the divorce." Mr. Mell said that he was fully agreeing to the divorce and to provide for his wife. Mr. Mell has a daughter, Emily, 22, who just finished Bucknell and moved to Virginia to teach first grade. His other daughter, Eliza, 20, is at Colby College in New York and wants to be a veterinarian. His son, Walter, 15, is a sophomore at Westminster School and "is very bright." Mr. Mell said that all of his children were supportive of him, but said that his wife would not speak with him. Asked about any family history of psychiatric or substance abuse disorders, Mr. Mell said that his father was alcoholic and depressed and that alcoholism permeated both of his parent's families.

Deviant and Non-Deviant Sexual History: Mr. Mell reached puberty at the age of 11. He had his first ejaculation at the age 13 by means of masturbation. His first crush was at 14 on a 14-year-old female. His first date was at 12 with a 12-year-old female. His first petting was at 14 with a 14-year-old female. His first genital experience was at 15 with a 15-year-old female. Mr. Mell first masturbated at the age of 13. The most orgasms he had had by any means in a week in his life was at 14. He had had five female sexual partners and no male sexual partners. He was heterosexual in his sexual orientation. The last time he masturbated was before his arrest, which was in April of 2018, and he was fantasizing about his wife, Kimberly. Asked if he had been physically abused during his childhood, he said that he had been hit several times by his father. He was never sexually abused. He thought that he had never had any erectile problems, but said that his wife at one time had suggested using an erectile facilitator, Cialis.

Regarding any paraphilic or hypersexual history, Mr. Mell denied to detailed questioning, exhibitionism, fetishism, frotteurism, pedophilia, sexual masochism, sexual sadism, transvestic fetishism, voyeurism, other paraphilias, ephebophilia (interest in adolescents), biastophilia (arousal or interest in rape), compulsive or excessive masturbation, promiscuity, use of pornography, engagement in telephone sex, cybersex (excessive sexual chatting over the Internet), sexual incompatibility, gender identity disorder, or other sexual

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disorders. He denied any other arrests or accusations of sexual harassment or of sexual crimes. He said that he was in 100% control, or in complete control, of his sexual urges and behavior.

Review of Systems: Was negative in detail for any psychiatric or somatic symptoms.

Psychological Testing: Included the following psychological tests with results as noted:

Tests to Assess Deviant and Non-Deviant Sexual Behavior:

1. **A Sexual SCID, or Structured Clinical Interview for Diagnoses for Sexual and Gender Identity Disorders:** which is a structured clinical interview to establish whether or not individuals may have diagnoses of sexual or gender identity disorders which is being developed at the Sexual Behavior Clinic at New York State Psychiatric Institute and in our private practice was administered. Mr. Mell did not make criteria for pedophilia or for any other paraphilic or hypersexual disorder.
2. **The Bancroft Self Report Scale of Sexual Interest and Activity (Developed by Bancroft, Tennent, Loucas, & Case, 1974):** was administered, with Mr. Mell being asked to score it with two points of time in mind. The first was before his arrest. The second was for the week before my interview of him. There are two components to this scale. The first is the Frequency of Sexual Thoughts Scale. This scale asks a patient to indicate how often he finds himself thinking sexy thoughts on a scale of 0, or no sexual thoughts at all, to 5, or sexual thoughts frequent and usually associated with feelings of sexual excitement in the previous week. Mr. Mell reported a 2 out of 5 for the first point in time before his arrest and a 0 out of 5 in the week before my interview of him. The second part asks how many ejaculations a patient had had in the week prior to the interview. Mr. Mell reported that he had had 1 ejaculation in the week period during the period before his arrest and no ejaculations in the week before I interviewed him. Both of these scores suggest a substantial reduction in sexual interest and behavior from the period of time before his arrest to the period just before my interview of him.
3. **The Clinical Global Impression Scale (Developed by Guy, 1976):** was scored, with Mr. Mell receiving a score of 1, or normal, not at all ill. This scale is a rating scale, which was developed at the National Institute of Mental Health for having clinicians rate how ill a patient is compared with the universe of patients that this clinician has seen with the sort of disorders that are in question. This scale ranges from 0, not assessed, or 1, normal, not at all ill, to 7, among the most extremely ill patients. Further, this scale has been used to estimate the degree of severity for a substantial number of disorders, ranging from schizophrenia, to

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eating disorders, to obsessive-compulsive disorder. In this case the focus of my estimation involved sexual and paraphilic disorders. In this case, I rated Mr. Mell as being a 1, or normal, not at all ill, compared with the thousand plus individuals with paraphilias and/or sexual disorders that I have evaluated.

4. **The Derogatis Interview for Sexual Function (Male Version):** was administered. This is a validated and normed instrument that makes enquiries about an individual's sexual behavior in a variety of domains for the time period of the 30 days prior to the administration of the test, including sexual cognition/fantasy; sexual arousal; sexual behavior/experiences; orgasms; and drive/relationship. Mr. Mell's scores were in the 2nd percentile for cognition/fantasy; 2nd percentile for arousal; 2nd percentile for sexual behavior/experience; 2nd percentile for orgasm; and 2nd percentile for sexual drive/relationship. Mr. Mell was in the 2nd percentile for men for overall sexual activity and functioning. These scores indicate a marked reduction in Mr. Mell's sexual drive since he was arrested and/or a significant depression.
5. **The Coleman Compulsive Sexual Behavior Inventory:** this is an inventory that focuses, among other things, on the inability to control sexual impulses. It has been described and validated in peer-reviewed literature. One scale of this instrument, the Control Scale, was used. This presents 13 items that an individual is asked to rate from 1, very frequent use, to 5, never engaged in. A higher score reflects greater control over sexual impulses. Pedophiles have a mean score of 36.2, sexual compulsives a mean of 31.7, and controls a mean of 57.2. Mr. Mell had a score of 65, suggesting that he was not sexually compulsive.
6. **The Kinsey Scale:** which is a scale developed by Kinsey which asks an individual to endorse a number from 0, indicating they were exclusively heterosexual to 6, indicating that they were exclusively homosexual. Mr. Mell circled a 0 indicating that he was exclusively heterosexual.

Tests to Screen for Other Psychiatric Syndromes:

7. **A SCID-I, or Structured Clinical Interview for DSM-IV and DSM-IV-TR Axis I Disorders:** which is a validated structured diagnostic instrument for the establishment of Axis I DSM-IV-TR psychiatric disorders. This resulted in diagnoses of major depressive disorder, single episode, in partial remission, panic disorder without agoraphobia, and anxiety disorder not otherwise specified.
8. **A Mini-Mental Status Examination:** which is a well-accepted screen for cognitive disorders. Mr. Mell received a score of 30 out of 30, suggesting no organic problems.

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9. **The Washton Alcohol and Drug Use Questionnaire:** which is a brief questionnaire which asks an individual to indicate if he or she had used drugs or alcohol, what the time frame of his or her use was, when he or she had last used any substances, and if substance use had ever been a problem. Mr. Mell indicated that he had used alcohol and cocaine and that he had never had any problem with these substances.
10. **The Michigan Alcohol Screening Test (MAST):** which is a 24 item validated screen for alcohol abuse. A score of 0 to 4 is considered non-alcoholic and Mr. Mell received a score of 1.
11. **The Drug Abuse Screening Test (DAST):** which is a 28 item validated screen for substance abuse. A score of less than 5 is normal. Mr. Mell received a score of 0, suggesting that he was not a substance abuser.
12. **The Adverse Childhood Experiences Scale:** which is a scale developed by Kaiser-Permanente and the National Institute of Health to quantify the degree of adverse childhood experiences an individual has experienced. It presents 10 questions, such as "did an adult or person at least 5 years older than you ever touch or fondle you?" This instrument has been validated and it has been demonstrated that the greater the score the more likely the occurrence of adverse physical or mental health consequences. Mr. Mell had a score of 3, which is a relatively high score, suggesting significant childhood adversity.

Tests to Assess Personality Functioning:

13. **The Hare Psychopathy Checklist, Full Version:** This is a system of screening for and assessing the degree of psychopathy that an individual has which I have had training in. The Hare asks a rater to make judgments on 20 factors, and can result in a maximal score of 40. I rated Mr. Mell as having a score of 9. This is a low score suggesting that he is not a psychopath.
14. **The Static-99R:** which is arguably the most validated and thoroughly studied risk assessment instrument for sexual recidivism. I computed Mr. Mell's score as being 0, which places him in the below average risk category for sexual recidivism.
15. **The Sexual Violence Risk-20:** which is an instrument to guide structured assessment of consideration and rating of a number of risk factors and other factors which would be relevant to the risk of sexual violence that an individual presents. I rated Mr. Mell as having 3 of 20 factors present. I would overall rate

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Mr. Mell's risk of sexual violence (i.e. sexual aggression towards an actual person) using this instrument as low.

16. **The SONAR:** which is an instrument developed by Hanson et al which attempts to incorporate dynamic factors into an individual's risk assessment. I computed Mr. Mell's score as being 2 (on a scale which extends from the lowest score of -4 to the highest score of 14). This falls within the low risk category (-4 to 3)
17. **The Level of Service/Case Management Inventory (LS/CMI):** which is an offender assessment developed by Andres, Bonta, and Wormith. It is used to assess an offender's needs and risks. It has been validated on 79,523 male community offenders in North America, including the United States. Mr. Mell received a score of 1 out of 43 (where a higher score is associated with greater needs and greater risks). Compared with individuals who are in the community, he is in the 2.5th percentile in terms of risk, meaning that 97.5% of individuals who are offenders who are now in the community had a higher risk of re-offense and greater need for services than Mr. Mell does.

Additional Data Relevant to the Evaluation:

1. **Telephonic Interview of Mr. Mell:** On September 18th, 2018, I spoke with Mr. Mell over the telephone to further clarify his use of drugs and sedatives. He said that he had been on Klonopin and Xanax since right after the helicopter accident as well as Xanax. He said that his regular medical doctor prescribed these. He was currently taking Klonopin, 1 mg tablets, one in the morning and two at night. He also was also on up to 6 1 mg tablets of Xanax. He said that at times he had taken 10 or 12 tablets in a day. He thought that he had at times used too much of these antianxiety agents.

Mental Status Examination: Mr. Mell presented on time for the appointment and was casually dressed. His affect was depressed and anxious. He denied hallucinations and there were no evident delusions or disorders of thought. He denied suicidal and homicidal ideation and intent currently and contracted to let his caregivers know if he developed this. He had good insight, good judgment, and good impulse control.

Impression: My impression is that Mr. Mell is a 52-year-old white male who has a history of growing up in an affluent and athletic family in New Jersey. This was disrupted by a bitter divorce when Mr. Mell was 12 or 13. Mr. Mell also had problems with learning and was diagnosed as having dyslexia at the age of 12 and treated extensively with therapy for his dyslexia for several years. He then managed to graduate from 9th grade and high school and college. He has an extensive history of achievement, being an avid athlete, engaging in soccer, hockey and lacrosse, working as an EMT, and working as a volunteer policeman.

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In 2011, Mr. Mell lost a friend and had a business partner who was severely injured in a helicopter accident; Mr. Mell was supposed to have accompanied his friends, but did not. He immediately felt guilty and has continued with survivor guilt since. The accident also resulted in litigation, which went on for several years. Associated with this, Mr. Mell developed a severe major depression, which continues into the present. This has been partially treated, but Mr. Mell is still significantly symptomatic from this.

Mr. Mell in May of 2018 was arrested by Federal authorities and charged with online enticement of a minor and traveling with the intent to engage in illicit sexual conduct. Subsequently, while in federal custody, he was charged by Hunterdon County authorities with sexual assault of a minor. Mr. Mell was released on bail and currently is under the supervision of Federal Probation. He is being treated with Klonopin and Xanax, both of which are being prescribed by his regular doctor. Mr. Mell feels that he has at times abused these benzodiazepines.

A thorough inventory of Mr. Mell's sexual history discloses no deviancies or hypersexual disorders. Specifically, Mr. Mell does not make criteria for pedophilia, which requires at least six months of sustained atypical sexual fantasy, urges, or behavior. Indeed, his sexual interest and behavior have been markedly attenuated since his arrest. Mr. Mell does have a significant depression, which began with the helicopter accident and continues, which has only partially been treated. Additionally, he continues with significant survivor guilt related to this accident. Mr. Mell's risk according to 5 actuarial instruments of another sexual crime is low or extremely low. This risk could be further reduced by sex offender specific therapy.

Additionally, Mr. Mell has for many years been treated for a fatal illness, primary sclerosing cholangitis, and has another very serious disease, Crohn's Disease, or regional enteritis, which is associated with a significantly increased risk of cancer and which requires frequent colonoscopies to surveil for the development of precancerous lesions.

Diagnoses (According to DSM-5):

1. Major Depression, Single episode, Severe, in partial remission
2. Generalized Anxiety Disorder
3. Sedative, Hypnotic, or Anxiolytic Use Disorder, moderate
4. Panic Disorder
5. Specific Learning Disorder with impairment in reading
6. Regional enteritis (Crohn's Disease)

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7. Primary sclerosing cholangitis
8. ? Allergy to penicillin

Opinion/Recommendations: Mr. Mell is not a pedophile and does not have any pattern of paraphilic or hypersexual behavior. His risk of a future sexual crime is low or very low according to 5 instruments used to assess such risk. This risk could be further reduced by his participation in sex offender specific therapy. Mr. Mell has 2 extremely severe medical disorders, Crohn's Disease and primary sclerosing cholangitis (PSC), both of which will require significant medical treatment and monitoring, and one of which has a high risk of fatality. Finally, recent literature has demonstrated (see appended article by Nunez et al.) that neither incarceration per se, nor the length of incarceration is associated with a decrease in risk of recidivism and this risk could be best managed and reduced through Mr. Mell's continuation in the community and in the therapeutic modalities he has developed.

I have included a copy of my resume for your information.

Sincerely,

A handwritten signature in black ink, appearing to read "Richard B. Krueger" followed by a stylized flourish.

Richard B. Krueger, M.D.

EXHIBIT B3

RICHARD B. KRUEGER, M.D.

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April 9th, 2019

Robert A. Bianchi, Esq.
Attorney at Law
The Bianchi Law Group, LLC
4 York Avenue, 2nd Floor
West Caldwell, New Jersey 07006

By Federal Express; telephone is: 862-210-8570

Re: Updated Evaluation of Mr. Steven Mell

Dear Mr. Bianchi:

I have updated my evaluation of Mr. Mell, with my report below:

Identifying Data & Chief Complaint: Mr. Mell is a 52-year-old white male who I evaluated extensively in September of 2018. At that time, he had been arrested by federal authorities for 3 counts of online enticement of a minor and 1 count of travel with intent to engage in illicit sexual conduct. I prepared an extensive psychiatric assessment and risk analysis in a report dated September 19th, 2019, which is appended. Mr. Mell was released on bail and has been living by himself in an apartment. He has been seeing a therapist, Dr. Daniel Water, on a weekly basis. Subsequent to the above charges, Mr. Mell pled guilty to interstate travel to engage in illicit sexual conduct and receipt of child pornography. He is scheduled to be sentenced and I was asked to update my evaluation given the fact of this guilty plea and these charges.

Sources of Information: Included the following interviews:

1. I recently interviewed Mr. Mell again for an hour on March 19th, 2019 in your offices
2. My prior interview of Mr. Mell for 4 hours on August 29th, 2018 and a follow-up telephone call on September 18th, 2018

It also included a review of the following materials:

1. My report of September 19th, 2018, which is appended

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2. A letter dated December 18th, 2018, referencing a plea agreement
3. A letter of November 21st. 2018, setting forth the plea agreement
4. A folder entitled "Extracted Photos-Phone Evidence No 2" with numerous jpg images, chats, and extraction report

Informed Consent for the Examination: Prior to the examination I informed Mr. Mell that I was performing an evaluation at the request of his defense attorney and that whatever he said to me could be included in a report to him or to her, and that his defense attorney may, in turn, choose to present this to the Court and the prosecution. I also presented information on the various psychological and written tests that I would be administering. Mr. Mell understood this information and signed appropriate consent forms.

Interim History: On March 19th, 2019, I met with Mr. Mell for an hour to review how he had been doing since I had evaluated him in September of 2018. He said, "I have had my ups and downs." He said that he has been continuing to receive antidepressant medication from his primary care doctor, Dr. Campbell. He was switched from Paxil to Prozac, taking a dose of 40 mg in the morning, in anticipation of incarceration in the federal prison system because of the availability of Prozac in the federal system and not Paxil. He said that he thought that his depression was much better. He said that he was increasingly anxious as his sentencing, which will be held on April 17th, 2019, approached. He reported that he had not abused sedatives or alcohol since I last saw him.

Mr. Mell said that all of his children were doing well. His 22-year-old daughter is teaching in Virginia. His 20-year-old daughter is on an elective in Sydney, Australia, and wants to go to veterinary school. His son is doing well. Mr. Mell has been seeing Dr. Daniel Water once a week in psychotherapy of a supportive nature. With respect to his depression he said when I saw him initially that he was very depressed, and that at one point he could hardly get out of bed. He said that now he was much less depressed and much more energetic.

Psychological Testing:

Tests to Assess Deviant and Non-Deviant Sexual Behavior:

1. **The Bancroft Self Report Scale of Sexual Interest and Activity (Developed by Bancroft, Tennent, Loucas, & Case, 1974):** was administered. Mr. Mell reported a score of 0 on the Frequency of Sexual Thoughts Scale. This scale asks the patient to indicate how often he finds himself thinking sexy thoughts on a scale of 0, or no sexual thoughts at all, to 5 or sexual thoughts frequent and usually associated with feelings of sexual excitement in previous week. It also asks how many ejaculations the patient has had in the week prior to the interview. Mr. Mell reported that he had had no ejaculations in the week before I interviewed him. Both of these scores suggest a continuing very low or absent sexual drive.

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2. **The Coleman Compulsive Sexual Behavior Inventory:** this is an inventory that focuses, among other things, on the inability to control sexual impulses. It has been described and validated in peer-reviewed literature. One scale of this instrument, the Control Scale, was used. This presents 13 items that an individual is asked to rate from 1, very frequent use, to 5, never engaged in. A higher score reflects greater control over sexual impulses. Pedophiles have a mean score of 36.2, sexual compulsives a mean of 31.7, and controls a mean of 57.2. Mr. Mell had a score of 65, suggesting that he was not sexually compulsive.

Tests to Assess Other Psychiatric Syndromes:

3. **The Beck Depression Inventory:** which is an instrument that is used to assess the degree of depressive symptomology that an individual has; the higher the score the more depressed an individual is. The maximum score is 63. Mr. Mell had a score of 6, which is a low score suggesting that he was not depressed currently.

Risk Assessment Instruments:

4. **The Child Pornography Offender Risk Tool (CPORT):** this is an instrument that has been developed in Canada and which has been validated. I did not administer it when I originally evaluated Mr. Mell because he had charges involving child pornography. The CPORT relies on scoring a number of demographic factors and aspects of the offending behavior; it results in a score of 0 to 7, the higher the score the greater the risk. Mr. Mell had a score of 1, placing him in the low risk group for sexual re-offense, involving either child pornography, contact sexual crimes, or both. The observed recidivism rate for individuals with this score at 5 years is 4.5%.

Mental Status Examination: Mr. Mell was casually dressed. He was coherent and able to provide a good history. There was no indication of thought disorder or psychosis. He denied hallucinations. He denied suicidal and homicidal ideation and intent. He reported no sexual interest in minor females and no compulsions to view pornography or child pornography, or to contact minors for sexual purposes. He was extremely remorseful for his criminal actions. He had good judgment, good impulse control, and good insight.

Impression: My impression is that Mr. Mell has dealt with his arrest, guilty plea, and supervision by federal probation very well. He has reported no impulses to engage in his offending behavior. His depression is quite markedly better, and, in my judgment, this is very likely because of his medication treatment with Prozac. Likewise, his anxiety is much better, again, because, in my judgment of the fluoxetine that he has been taking.

Diagnoses: (According to DSM-5):

1. Major Depression, Single episode, Severe, in complete remission

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2. Generalized Anxiety Disorder, in complete remission
3. Sedative, Hypnotic, or Anxiolytic Use Disorder, moderate, in remission in an uncontrolled environment
4. Panic Disorder, in remission
5. Specific Learning Disorder with impairment in reading
6. Regional enteritis (Crohn's Disease)
7. Primary sclerosing cholangitis
8. ? Allergy to penicillin

Opinion/Recommendations: Mr. Mell is not a pedophile and does not have any pattern of paraphilic or hypersexual behavior. His risk of a future sexual crime is low or very low according to 6 instruments used to assess such risk and will be reduced by both therapy and supervision when he is in the community.

Mr. Mell has 2 extremely severe medical disorders, Crohn's Disease and primary sclerosing cholangitis (PSC), both of which will require significant medical treatment and monitoring, and one of which has a high risk of fatality. He should have these fully evaluated before going to prison and his incarceration should take into account the medical necessity of monitoring these two disorders. He also has a history of sedative abuse and in my judgment would be an excellent candidate for the Residential Alcohol and Treatment Program (RDAP) in the federal prison system.

Sincerely,

Richard B. Krueger, M.D.

Date CV Prepared April 11th, 2018

Personal Data

Name	Richard Bohn Krueger, M.D.
Office Address	Medical Director Sexual Behavior Clinic New York State Psychiatric Institute & Columbia University Department of Psychiatry, Unit #45 1051 Riverside Drive New York, N.Y. 10032
Private Practice	210 East 68th Street, Suite 1-H New York, New York 10065-6024
Telephone	(212) 740-7330 (New York State Psychiatric Institute) (212) 740-7341 (New York State Psychiatric Institute) (212) 517-6624 (private office) (212) 517-4073 (private office fax) (917)-750-1596 (cell phone anywhere, anytime)
Email	rbk1@cumc.columbia.edu
Birthdate	7/23/45

Academic Training

1967 B.A.	Albion College, Albion, MI
1977 M.D.	Harvard Medical School, Boston, MA

Traineeship

1977-1978	Internship in Medicine, Boston VA Hospital, Boston, MA
1978-1980	Junior and Senior Residency, Medical, Boston VA Hospital, Boston, MA
1980-1983	Residency in Psychiatry, Department of Psychiatry Boston University, Boston, MA

Licensure

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1977-Present	Narcotic License #AK 961-0684
2006-2008	Buprenorphine License #XK 961-0684
2006-6008	New York State Buprenorphine Authorization #000646
1989-1991; 1997-Present	New Jersey Registration # 25MA05309800
1989-Present	New York License Registration #177371
1995-Present	New York State Worker's Compensation #177371-2

Board Certification

1980	American Board of Internal Medicine, #74112
1984	American Board of Psychiatry and Neurology, Inc., in Psychiatry, Certificate #26401
1996	American Board of Psychiatry and Neurology, Inc., Added Qualifications in Forensic Psychiatry, Certificate #382, recertified through 2026
1997	American Board of Psychiatry and Neurology, Inc., Added Qualifications in Addiction Psychiatry, Certificate #1183, certified through 2026

Professional Organizations and Societies

1978-Present	American Medical Association
1978-Present	Massachusetts Medical Society
1981-Present	American Psychiatric Association
1981-1989	Massachusetts Psychiatric Association
1985-Present	American Academy of Psychiatry and the Law
1989-Present	New York Psychiatric Society
1991-Present	New York Academy of Science

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Page 3

1993-Present	Association for the Treatment of Sexual Abusers
1996-Present	American Academy of Forensic Sciences
2002-Present	Society for Behavioral Neuroendocrinology
2002-Present	International Academy of Sex Research
2003-Present	American Association for the Advancement of Science
2004-Present	International Association of Forensic Mental Health Services
2004-Present	American Psychopathological Association
2004-Present	The Society for the Advancement of Sexual Health
2006-Present	International Academy of Law and Mental Health
2006-Present	International Association for the Treatment of Sexual Offenders
2010-Present	Education Committee, American Academy of Psychiatry and the Law
2015-Present	Member, American Academy of Addiction Psychiatry
2016-Present	Member, American College of Psychiatrists

Academic Appointments

1983-1989	Clinical Instructor in Psychiatry, Boston University Medical School
1985-1989	Clinical Instructor in Psychiatry, Harvard Medical School
1989-1998	Assistant Clinical Professor of Psychiatry, Columbia University, College of Physicians and Surgeons
1998-Present	Associate Clinical Professor of Psychiatry, Columbia University, College of Physicians and Surgeons

Hospital Appointments

1982-1987	Consulting Psychiatrist, Hampstead Hospital, Hampstead, NH
1983-1984	Staff Psychiatrist, Westwood Lodge Hospital, Westwood, MA

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Page 4

1984-1988	Staff Psychiatrist, Pembroke Hospital, Pembroke, MA
1985-1986 Hospital	Assistant Psychiatrist, McLean Hospital/Bridgewater State Program, Bridgewater, MA
1985-1989	Consulting Psychiatrist, Massachusetts Treatment Center, Bridgewater State Hospital, Bridgewater, MA
1985-1989	Assistant Attending Psychiatrist, McLean Hospital, Belmont, MA
1988-1989	Attending Psychiatrist, Waltham/Weston Hospital Waltham, MA
1989-1998	Assistant Attending Psychiatrist, Columbia Presbyterian Medical Center, New York, NY
1990-2000	Attending Psychiatrist, Gracie Square Hospital, New York, NY
1990-2000	Attending Psychiatrist, Lenox Hill Hospital, New York, NY
1991-1994	Research Psychiatrist II, Department of Biological Psychiatry, New York State Psychiatric Institute, New York, NY
1991-1994	Member, Department of Biological Psychiatry, New York State Psychiatric Institute, New York, NY
1991-1994	Research Psychiatrist II, New York State Psychiatric Institute, New York, NY
1991-Present	Attending Psychiatrist, New York State Psychiatric Institute, New York, NY
1994-1998	Clinical Psychiatrist II, Inwood Mental Health Clinic, Washington Heights Community Service, New York State Psychiatric Institute, New York, NY
1995-Present	Medical Director, Sexual Behavior Clinic, New York State Psychiatric Institute, New York, NY
1998-Present	Clinical Specialist II, Sexual Behavior Clinic, New York State Psychiatric Institute, New York, NY
1998-Present	Attending Psychiatrist, New York Presbyterian Hospital, New York, NY

Honors

1966	<i>Phi Beta Kappa</i>
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1966	<i>Omicron Delta Kappa</i>
1967-1968	Rotary International Scholarship for study at University of Stockholm, Sweden
1983	John Murray Prize for Research in Psychiatry, Department of Psychiatry, Boston University Boston, MA
2004	Distinguished Alumni Award, Albion College, Albion, MI
2007	Distinguished Service Award from the New York State Alliance of Sex Offender Service Providers and the New York State Chapter of the Association for the Treatment of Sexual abusers presented "in appreciation of your many years of service. We honor you for your dedication, leadership, clinical expertise and scholarly contributions to the field of sex offender management" NYSATSA/NYSASOSP May 3 rd , 2007, Poughkeepsie, NY
2014	Life Fellow, American Psychiatric Association
2017	Distinguished Life Fellow, American Psychiatric Association
2017	Society for the Advancement of Sexual Health Carnes Award

Fellowship and Grant Support

1990-1994	Co-Investigator, NIMH, (MH35636) 09-13 Affective and cognitive consequences of ECT 1990-1995 (subject to MERIT award extension to 2000). \$1,008,144 direct costs (90-95). Principal Investigator Harold A. Sackeim
1992-1993	Principal Investigator, Cambridge Neuroscience (#CNS-1879-001). A placebo-controlled study of pramiracetam in depressed inpatients undergoing electroconvulsive therapy
1992-1993	Principal Investigator, Cambridge Neuroscience (#CNS-1879-002). A follow-up study of patients completing protocol (#CNS-1879-001) (a placebo- controlled study of pramiracetam in depressed inpatients undergoing electroconvulsive therapy)
1992-1994	Co-Investigator, NIMH, (MH47739) 01-05 Continuation pharmacotherapy following ECT. (NYSPI site \$759,101 direct costs all sites \$2,490,284). Principal Investigator Harold A. Sackeim.

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1993-1994 Co-Investigator, Dana Foundation Research Program on Age-Related Memory Defects (ECT memory project). \$250,000 direct costs. Principal Investigators Eric Kandel and Lewis P. Rowland.

Other Grants and Contracts

11/2000-9/2001 Contract No 0208-2001-07 (MH) with Department of Probation of the United States District Court for the Southern District of New York; contract for the evaluation and care of federal offenders convicted of sexual crimes.

5/2001-9/2001 Non-competitive Purchase Order #50668A with the Department of Probation of United States District Court for the Southern District of New York for to administer penile plethysmograph and polygraph examinations.

10/2001-9/2002 Non-competitive Purchase Order #56640 with the Department of Probation of United States District Court for the Southern District of New York to administer Individual and Group Mental Health Counseling/Sex Offender.

2006-2007 Consultation Agreement with GalaxoSmithKlein regarding development of methods and scales to assess for pathological sexual behavior, pathological gambling, and other disorders of impulse.

2007-2008 New York State Office of Mental Health Policy Scholar Grant for 2007-2008 academic year, for project entitled ""An Analysis of Treatment Response and Medication Satisfaction in a Group of Patients Treated with Gonadotropin Releasing Hormones Agonist Therapy for Hypersexual and/or Paraphilic Disorders and An Analysis of Diagnoses of a Group of Patients Arrested for Crimes Against Children over the Internet." Amount awarded: statistical, data entry, and research design support provided by the New York State Office of Mental Health.

Departmental and University Committees

1992-1995 NYSPI-Columbia University IRB Psychopharmacology I Subcommittee, member

1992-1995 New York State Psychiatric Institute--Columbia University Department of Psychiatry Institutional Review Board, member

Teaching Experience and Responsibilities

1977-1980	Clinical Instructor in Medicine, Tufts University School of Medicine (taught medical students, interns, and residents in medicine)
1977-1980	Clinical Instructor in Medicine, Boston University, School of Medicine (taught medical students, interns, and residents in medicine)
1980-1983	Clinical Instructor in Psychiatry, Department of Psychiatry, Boston University (taught medical students and residents in psychiatry)
1983	Taught seminars in clinical interviewing using videotape for medical students and psychiatric residents at Boston University
1983-1984	Taught Boston University medical students during their psychiatry rotation at Westwood Lodge Hospital
1989-1991 1994-2000	Faculty, Psychiatric Medicine I and Psychiatric Medicine II, teaching Columbia first and second year medical students
1991-1993	Supervisor PGY IV Residents in psychiatry at Columbia P&S in consultation and liaison psychiatry
1991-1994	Supervisor and instructor, Columbia medical students and psychiatric residents in performing ECT
1991-1994	Developed and directed a Continuing Medical Education Fellowship on Electroconvulsive Therapy offered by the Department of Psychiatry of Columbia P&S and the Department of Biological Psychiatry of New York State Psychiatric Institute
1994-1997	Clinical Supervisor, Columbia psychology interns
1994-1998	Supervisor, Columbia PGY III medical students in their outpatient experience during their psychiatry rotation
1994-2005	Faculty, Harlem Hospital Residency in Psychiatry, teaching PGY II residents on sexual disorders and paraphilias and their treatment
1994-present	Evaluation supervisor, Columbia PGY III psychiatric residents
1996-present	Faculty, Creedmoor Psychiatry Residency, teaching PGY III psychiatric residents on sexual disorders and their treatment
1999-present	Faculty, Columbia Cornell Fellowship in Forensic Psychiatry

Other Professional Activities

1984-1989	Consulting Psychiatrist, Blue Cross/Blue Shield, Utilization Review Program
1984-1989	Consulting Psychiatrist, The Neurologic Center at Forest Manor, Middleborough, MA
1984-1989	Consulting Psychiatrist, Jordan Hospital, Plymouth, MA
1985-1989	Consulting Psychiatrist, The Boston Neurobehavioral Center, Boston, MA
1985-1989	Consulting Psychiatrist, The Massachusetts Treatment Center, Bridgewater, MA
1988-1989	Consulting Psychiatrist, McLean Health Services, Belmont, MA
1988-1989	Consulting Psychiatrist, Framingham Court Clinic, Framingham, MA
1989-1998	Consulting Psychiatrist, Sexual Behavior Clinic, New York State Psychiatric Institute, New York, NY
1989-1991	Attending Psychiatrist, Inpatient Psychiatric Unit, Allen Pavilion, Columbia Presbyterian Medical Center
1994	Guest Reviewer, <i>Biological Psychiatry</i>
1995-Present	Member, Sexual Offenders Committee, American Academy of Psychiatry & the Law
1995-Present	Authorized Provider, State of New York Worker's Compensation Board
1995-Present	Expert Consultant, New York State Health Department's Office of Professional Medical Conduct (OPMC), Albany, NY
1996-Present	Expert Consultant, New Jersey State Board of Medicine Trenton, New Jersey
1996-2002	Member, Board of Directors, The New York Academy of Traumatic Brain Injury, Inc.

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1997-2001	Member, Psychopharmacology Committee, American Academy of Psychiatry & the Law
1997-Present	Member, Board of Directors, New York State Chapter, Association for the Treatment of Sexual Abusers
1998-2000	Consultant, NY State Office of Mental Health/Sidney Albert Institute of the Parsons Child and Family Center Workgroup on Developing a Statewide System of Care for Child and Adolescent Sexual Abusers
1998-2001	Chairman, Psychopharmacology Committee, American Academy of Psychiatry & the Law
1999-2002	Consultant, NY State Office of Mental Health Task Force on Sex Abuse
2000	Guest Reviewer, <i>Psychiatry Research</i>
2001	Guest Reviewer, <i>Practical Psychiatry and Behavioral Health</i>
2003	Guest Reviewer, <i>Progress in Neuro-Psychopharmacology & Biological Psychiatry</i>
2005	Guest Reviewer, <i>European Archives of Psychiatry and Clinical Neuroscience</i>
2005-2011	Vice-President, New York State Chapter, Association for the Treatment of Sexual Abusers
2007	Guest Reviewer, <i>Sexual Abuse</i>
2007	Guest Reviewer, <i>Journal of Nervous and Mental Disease</i>
2008-2011	Member, The Trauma and Stress Committee, American Academy of Psychiatry and the Law
2008-Present	Member, DSM-V, Sexual and Gender Identity Disorders Work Group
2008-Present	Expert Reviewer, <i>Journal of Sex Research</i>
2009-Present	Expert Reviewer, <i>Journal of Sexual Medicine</i>
2009-Present	Member, Society for Sex Therapy and Research (SSTAR)
2009	Guest Reviewer, <i>Harvard Review of Psychiatry</i>

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2010	Guest Reviewer, <i>CNS Spectrums</i>
2011	Guest Reviewer, <i>Sexual Abuse</i>
2011	Guest Reviewer, <i>Neuropsychiatry</i>
2011	Guest Reviewer, <i>Sexuality & Culture</i>
2011	Guest Reviewer, <i>Journal of Abnormal Psychology</i>
2011-Present	Reviewer, <i>Journal of Sex and Marital Therapy</i>
2011-Present	Reviewer, <i>Journal of Sexual Medicine</i>
2011-Present	Member, Education Committee, American Academy of Psychiatry and the Law
2011-Present	Member, Sexual Health and Disorders Committee of the World Health Organization; this committee is composed of international authorities and is making recommendations for revision of the sexual disorders section for ICD-11 (The International Classification of Diseases-11 th Edition)
2012	Guest Reviewer, <i>Archives of Sexual Behavior</i>
2012-2013	President Elect, New York State Association for the Treatment of Sexual Abusers
2013-2015	Elected by Board to position of President, New York State Association for the Treatment of Sexual Abusers
2013	Guest Reviewer, <i>Journal of Psychiatric Research</i>
2013	Guest Reviewer, <i>Archives of Sexual Behavior</i>
2013	Participant, Meeting on Protocol Development for Field Testing of ICD-11 Sexual Disorders and Sexuality-Related Conditions
2013	Guest Reviewer, <i>International Journal of Family Medicine</i>
2013	Guest Participant, Human Sexuality Committee, Group for the Advancement of Psychiatry, White Plains, New York
2014	Guest Reviewer, <i>Sexual Medicine Reviews</i>
2014	Guest Reviewer, <i>Archives of Sexual Behavior</i>

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2014	Guest Reviewer, <i>Journal of Forensic Psychiatry and Psychology</i>
2014	Guest Reviewer, <i>Journal of Psychiatry and Neuroscience</i>
2014	Guest Reviewer, <i>Journal of Sex and Marital Therapy</i>
2014	Guest Reviewer, <i>Journal of Sex Research</i>
2014	Guest Reviewer, <i>Comprehensive Psychiatry</i>
2015	Guest Reviewer, <i>Archives of Sexual Behavior</i>
2015	Guest Reviewer, <i>Journal of Sex and Marital Therapy</i>
2015	Guest Reviewer, <i>Plos One</i>
2015	Guest Reviewer, <i>Comprehensive Psychiatry</i>
2015	Guest Reviewer, <i>Journal of Forensic Psychiatry and Psychology</i>
2015	Guest Reviewer, <i>Journal of Sex Research</i>
2015	Guest Reviewer, <i>Behavioral Sciences</i>
2015	Invited participant, Assembly Standing Committee on Correction, Public Roundtable on Sex Offender Residency Restrictions to examine the efficacy of sex offender residency restrictions and identify best practices to promote public safety and prevent recidivism of sex offenders. Wednesday, May 20 th , 2015, Legislative Office Building room 711-A, 7 th Floor, Albany, New York 12248 called by Daniel J. O'Donnell, Member of the Assembly, Chairperson, Committee on Correction
2016	Guest Reviewer, <i>Journal of Sex Research</i>
2016	Guest Reviewer, <i>Addiction</i>
2016	Member, American College of Psychiatrists
2016	Guest Reviewer, <i>International Journal of Psychological Research</i>
2016	Guest Reviewer, <i>Medical Journals</i>
2016	Member, DSM Review Committee for Externalizing Disorders and Personality Disorders (Personality, Disruptive, Impulse Control,

	Conduct Disorders, Substance Use Disorders, Paraphilic Disorders)
2017	Guest Reviewer, <i>Journal of Psychopharmacology</i>
2017	Guest Reviewer, <i>American Journal of Psychiatry</i>
2017	Guest Reviewer, <i>The Journal of the American Academy of Psychiatry and the Law</i>
2017	Guest Reviewer, <i>Journal of Sex and Marital Therapy</i>
2017	Guest Reviewer, <i>Development and Psychopathology</i>
2017	Guest Reviewer, <i>Journal of Sexual Medicine</i>
2018	Guest Reviewer, <i>International Clinical Psychopharmacology</i>
2018	Guest Reviewer, <i>Journal of Psychiatric Practice</i>
2018	Guest Reviewer, <i>Journal of Sex and Marital Therapy</i>
2018	Guest Reviewer, <i>Journal of Sexual Medicine</i>
2018	Guest Reviewer, <i>Journal of Forensic Research</i>

Bibliography**PEER REVIEWED ARTICLES**

- 1984**
1. Krueger RB, Levy E, Fox B, Cathgart E, Black P: Lymphocyte subsets in patients with major depression Preliminary findings. *Advances*. 1984; 1:5-9
- 1985**
2. Levy E, Krueger RB: Depression and the immune system. *Directions in Psychiatry*. 1985; 5:1-8
- 1992**
3. Krueger RB, Sackeim HA, Gamzu ER: Pharmacological treatment of the cognitive side effects of ECT: A review. *Psychopharmacology Bulletin*. 1992; 28:409-424
- 1993**

4. Krueger RB, Fama JM, Devanand DP, Prudic JP, Sackeim HA: Does ECT permanently alter seizure threshold? *Biological Psychiatry*. 1993; 33:272-276
- 1994**
 5. Malaspina D, Devanand DP, Krueger RB, Prudic J, Sackeim HA: The significance of clinical EEG abnormalities in depressed patients treated with ECT. *Convulsive Therapy*. 1994; 10:259-266
 6. Prudic J, Sackeim HA, Devanand DP, Krueger RB, Settembrino JM: Acute cognitive effects of subconvulsive electrical stimulation. *Convulsive Therapy*. 1994; 10:4-24
 7. Devanand DP, Krueger RB: Electroconvulsive therapy in the elderly. *Current Opinion in Psychiatry*. 1994; 7:359-364
- 1998**
 8. Krueger RB, Glancy GD, Bradford JM: The Abel screen: A new instrument for assessing sexual interest. *J Am Acad Psychiatry Law*, Vol 26, No. 2, 1998; 277-280
- 2000**
 9. Krueger RB, Kaplan MS: Disorders of Sexual Impulse Control in Neuropsychiatric Conditions. In T. W. McAllister, Guest Ed. *Seminars in Clinical Neuropsychiatry Pharmacological of Neuropsychiatric Syndromes*. Vol. 5, No. 4, 2000; pp. 266-274
- 2001**
 10. Krueger RB, Kaplan MS: Depo-leuprolide acetate for the treatment of the paraphilias: A report of 12 cases. *Archives of Sexual Behavior*. 2001; 30:409-422
 11. Krueger RB, Kaplan MS: The paraphilic and hypersexual disorders: An Overview. *Journal of Psychiatric Practice*. November 2001; 7:391-403
- 2002**
 12. Krueger RB, Kaplan MS: Behavioral and psychopharmacological treatment of the paraphilic and hypersexual disorders. *Journal of Psychiatric Practice*. January 2002; 8:21-32
 13. Krueger RB, Kaplan MS: Treatment resources for the paraphilic and hypersexual disorders. *Journal of Psychiatric Practice*. January 2002; 8:59-60
 14. Krueger RB, Kaplan MS: A favorable view of the DSM-IV diagnosis of pedophilia and empathy for the pedophile. *Archives of Sexual Behavior*. December 2002; 31:486-488
- 2003**

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15. Krueger RB: A Positive View of Spitzer's Research and an Argument for Further Research. Archives of Sexual Behavior. October 2003; 32:443-444

2006

16. Shajnfeld A, Krueger RB: Integrating Law and Psychiatric Practice: Reforming Non-Punitive Responses to Sexual Offending. Developments in Mental Health Law July 2006; 25: 81-99.

2009

17. Krueger RB, Kaplan, M, First, M: Sexual and other axis I diagnoses of 60 males arrested for crimes against children involving the Internet, CNS Spectrums, Volume 14, No. 11, 2009; 623-631.

2010

18. Krueger RB: The DSM Diagnostic Criteria for Sexual Sadism: Archives of Sexual Behavior, Volume 39, No. 2, 2010, 325-345.
19. Krueger RB: The DSM Diagnostic Criteria for Sexual Masochism: Archives of Sexual Behavior, Volume 39, No. 2, 2010; 346-356.
20. Kaplan MS, Krueger RB: Diagnosis, Assessment, and Treatment of Hypersexuality: Annual Review of Sex Research, Journal of Sex Research, Vol 47, Issue 2-3; 2010; 181-198.
21. Krueger RB: DSM-V Options: Sexual Sadism, Sexual Masochism, and Paraphilic Coercive Disorder: Forum for the Association for the Treatment of Sexual Abusers, Vol XXII, Issue 1, 2010, 1-8.

2011

22. Krueger, RB: Critical Appraisals of the Proposed DSM-5 Paraphilia Diagnoses: The Journal of the American Academy of Psychiatry and the Law; Vol 39, 2011; 237.

2012

23. Krueger, RB, Kaplan, MS: Paraphilic Diagnoses in DSM-5: The Israeli Journal of Psychiatry and Related Sciences; Vol 49, No. 4, 2012, 248-254.
24. Kaplan, M, Krueger RB: Cognitive-Behavioral treatment of the paraphilias: The Israeli Journal of Psychiatry and Related Sciences; Vol 49, No. 4, 2012, 291-296.

2013

25. Krueger, RB, Weiss, SL, Kaplan, MS, Braunstein, LJ, Wiener, E: The Impact of Internet Pornography Use and Cybersexual Behavior on Child Custody and Visitation; *Journal of Child Custody*, 10: 1-31, 2013, DOI: 10.1080/15379418.2013.781860

2016

26. Turner, D, Schottle, D, Krueger, R, Briken, P: Sexual Behavior and its correlates after traumatic brain injury; *Current Opinion in Psychiatry*, 28: 1-8, 2015, DOI:10.1097/YCO.0000000000000144
27. Reed, GM, Drescher, J, Krueger, RB, Atalia, E, Cochran, SD, First, MV, Cohen-Kettenis, PT, Arango-de Montis, I, Parish, SJ, Cottler, S, Briken, P, Saxena, S: Disorders related to sexuality and gender identity in the ICD-11: revising the ICD-10 classification based on current scientific evidence, best clinical practices, and human rights consideration. *World Psychiatry* 15: 3, October 2016, 205-221

- 2017 28. Krueger, RB, Reed, GM, First, MB, Marais, A, Kismodi, E, Briken, P (2017): Proposals for paraphilic disorders in the International Classification of Diseases and Related Health Problems, Eleventh Revision. *Archives of Sexual Behavior* DOI: 10.1007/s10508-017-0944-2; volume 46: 1529-1545.

29. Turner, D, Petermann, J, Harrison, K, Krueger, R, Briken, P (2017) Pharmacological treatment of patients with paraphilic disorders and risk of sexual offending: An international perspective. *The World Journal of Biological Psychiatry*: DOI: 10.1080/15622975.2017.1395069

- 2018 30. Briken, P., Boetticher, A., Kismodi, E., Reed, G.M. ICD-11 Proposals for paraphilic disorders-Current legal situation and implications for Germany (in preparation).
31. Krueger, R. Kaplan, M. Treatment response and medication satisfaction in a group of males treated with gonadotropin releasing hormone agonist therapy for hypersexual and/or paraphilic disorders (in preparation)
32. Krueger, R. Kaplan, M. Sexual and psychiatric diagnoses in a group of 200 males arrested for crimes against children over the internet (in preparation)

CASE REPORTS

2006

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1. Krueger RB, Hembree W, Hill M (2006). Prescription of Medroxyprogesterone Acetate to A Patient with Pedophilia, Resulting in Cushing's syndrome and Adrenal Insufficiency. *Sexual Abuse: A Journal of Research & Treatment*; 18:227-228.
2. Krueger RB, Kaplan MS (2006). "I Want to Try It. What Else Can I Do?" Chemical Castration as a Treatment for Pedophilia. R Spitzer, J. Williams, M. First (Eds). DSM-IV-TR Casebook, Volume 2. American Psychiatric Association Press. Washington DC. 309-334. (translated into Hungarian, APA Press, 2007).
- 2018** 3. Kaplan, M, Krueger, R. Cybersexual Disorder: A case report. (in preparation)
4. Krueger, R, Kaplan, M. Reduction in aggression in a male treated with a gonadotropin releasing hormone agonist for hypersexual behavior (in preparation)

CHAPTERS AND BOOKS

1985

1. Levy EM, Krueger RB. (1985) Depression and the immune system. In Flach, F. (Ed.) Directions in Psychiatry. Hatherleigh Company, Ltd., New York, pp. 1-8

1988

2. Levy EM, Krueger RB. (1988) Depression and the Immune System. In F. Flach (Ed.) Affective Disorders. Norton Publishers, New York, pp. 186-198

1995

3. Krueger RB, Sackeim HA. (1995) ECT and Schizophrenia. In S. Hirsch and D. Weinberger (Eds.). Schizophrenia. Blackwell Scientific Publications Ltd., Oxford, pp. 503-545

1996

4. Levy E, Chancellor-Freeland C, Krueger RB. (1996) Depression and the Immune System: The Hatherleigh Guide to Managing Depression. Hatherleigh Press, New York, pp. 15-32

1997

5. Krueger RB, Kaplan MS. (1997) Frotteurism Assessment and treatment. In D. R. Laws and W. O'Donohue (Eds.). Sexual Deviance: Theory, Assessment, and Treatment. Guilford Press, New York, pp. 131-151
6. Kaplan M, Krueger RB. (1997) Voyeurism: Psychopathology and Theory. In D. R. Laws and W. O'Donohue (Eds.). Sexual Deviance: Theory, Assessment, and Treatment. Guilford Press. New York, pp. 297-310

2000

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7. Krueger RB, Kaplan MS. (2000) The non-violent serial offender: Exhibitionism, frotteurism, and telephone scatologia. In L.B. Schlesinger (Ed.) Serial Offenders Current Thought, Recent Findings, Unusual Syndromes. CRC Press, Boca Raton, FL., pp. 103-118
8. Krueger RB, Kaplan MS. (2000) Evaluation and Treatment of Sexual Disorders: Frottage. In VandeCreek, L. (Ed.) Innovations in Clinical Practice: A Source Book. Vol. 18, Professional Resource Press. Sarasota, FL. pp. 185-198

2003

9. Kaplan MS, Krueger RB. (2003) Adolescent Sex Offenders. In Rosner, R. (Ed.) Principles and Practice of Forensic Psychiatry. Second Edition. Chapter 45, Arnold, London. pp. 455-462

2006

10. Krueger RB. (2006) A Positive View of Spitzer's Research and an Argument for Further Research. In J. Drescher and K.J. Zucker (Eds.) Ex-Gay Research: Analyzing the Spitzer Study and Its Relation to Science, Religion, Politics, and Culture. Harrington Park Press. Binghamton, New York. pp. 151-152

2008

11. Krueger RB, Kaplan MS. (2008) Frotteurism: Assessment and treatment. In D. R. Laws and W. O'Donohue (Eds.). Sexual Deviance: Theory, Assessment, and Treatment. 2nd Edition. Guilford Press, New York. pp. 150-163.
12. Krueger RB. (2008) Legislation Must Be Based On Scientific Research. In Zott, LM (Ed.) Sex Offenders and Public Policy. Current Controversies. Greenhaven Press. Thomson Gale. New York. pp. 139-143.

2009

13. Krueger RB, Wexler R, Kaplan M, Saleh F. (2009) Orchiectomy. In Saleh, FM, Bradford, Grudzinskas, AJ, Bradford JM, Brodsky DJ. (Eds.). Sex Offenders. Oxford University Press, New York, New York. pp. 171-188.

2016

14. Krueger, RB, Kaplan MS. (2016) Non-Contact Paraphilic Sexual Offenses. In Hoberman, H, Phoenix, A. (Eds.). Sexual Offending: Predisposing Antecedents, Assessments, & Management. Springer, New York. pp 79-102.

15. Attala E, Cochran SD, Drescher J, Krueger RB, Reed, G (2015) Sexual Disorders and Sexual Health. In Robert MC (Ed) The Evidence Base for ICD-11. American Psychological Association, Washington, D.C. In press.
16. Krueger, RB, Kaplan, MS (2016) Paraphilic Disorders. In Tasman, A, Kay, J, Lieberman, JA, First, MB, Riba, MB. Psychiatry. Fourth Edition. John Wiley & Sons, Ltd. Hoboken, New Jersey, pp. 1749-1758.

2018

17. Attala, E., Cochran SD, Drescher, J., Krueger, R, & Reed, G. Sexual disorders and sexual health. in: Roberts, M.C, Reed, G, Eds. The Evidence Base for ICD-11. Washington, D.C.: American Psychological Association; in press.

REVIEWS, EDITORIALS, AND LETTERS

1991

1. Krueger RB, Silver JM. Reply to Safe Administration of ECT in a patient with a calcified frontal mass (letter). *Journal of Neuropsychiatry*, 1991; 3:354

1992

2. Sackeim HA, Prudic J, Devanand DP, Krueger RB. In reply: stimulus dosing strategies and the efficacy of unilateral ECT. (letter) *Convulsive Therapy*, 1992; 8:46-52

1994

3. Krueger RB, Devanand DP, Prudic J, Sackeim HA. Response to Tomasson, Winockur and Pfohl (letter) *Biological Psychiatry*, 1994; 35:427-429

2005

4. Krueger RB, Kaplan MS. A Review of "Clinical Management of Sex Addiction" edited by Patrick J. Carnes and Kenneth M. Adams. New York Brunner-Routledge, 2002. *Journal of Sex & Marital Therapy*. October-December 2005; 5:448-450

2006

5. Krueger RB. A Review of "Identifying and Treating Sex Offenders. Current Approaches, Research, and Techniques." A Co-Publication as a Printed Volume of a Volume of the *Journal of Child Sexual Abuse*. Vol 12, No. 3/4 2003. *Psychiatric Services*. February 2006; 57: 282
6. Krueger RB. "New York Should Not Pass a Violent Predator Law Without Further Study." *The Alliance. NYASASOP/NYATSA*. Winter 2005/2006. Volume 8, Issue 1: 4.
7. Krueger RB. "Dear NYATSA and Alliance Members." *The Alliance. NYASASOP/NYATSA*. Fall 2006. Volume 8, Issue 3: 1.

8. Krueger RB. Board of Directors, NYSATSA & NYASASOP. "NYS ATSA and Alliance Policy Statement Concerning Sexual Violent Predator Legislation." The Alliance. NYASASOP/NYATSA. Fall 2006. Volume 8, Issue 3: 2-3.
9. Krueger RB. Board of Directors, NYSATSA & NYASASOP. "NY State ATSA Letter to Members of the New York State Assembly, Senate, and Joint Conference Committee, and Others Regarding Proposed Sexually Violent Predator (SVP) Legislation." The Alliance. NYASASOP/NYATSA. Fall 2006. Volume 8, Issue 3: 4-5.
10. Krueger RB. Vice-President, NYSATSA. "Toward Treatment for Sexual Abusers." Letter to the Editor, The New York Times, December 31, 2006. Commentary on Editorial "Rush to Judgment on Sex Offenders" December 10, 2006, New York Times.

2007

11. Krueger RB. "The New American Witch Hunt." Invited op-ed. March 11, 2007, Los Angeles Times.
12. Krueger, RB. Vice-President NYSATSA. "Dear Colleague" Letter in The Alliance, Summer 2007, Volume 9, Issue 3, 5-6.
13. Krueger, RB. Lau, KL. for the Boards of the NYSATSA. "Position Paper Regarding the Sex Offender Management and Treatment Act of 2007." Filed and Bound to Article 10 of the Mental Hygiene Law (10.01-10.17), April 2007, and published in The Alliance, Summer 2007, pages 2-4.

2010

14. Kaplan, MS, Krueger, RB. "A Review of 'Human sexuality and its problems (3rd. ed.), Journal of Sex and Marital Therapy, 37:1 72-73.

2011

15. Kafka, M. P., & Krueger, R. B. (2011a). Response to Moser's critique of Hypersexual Disorder for DSM-5. *Archives of Sexual Behavior*, 40, 231-232.
16. Kafka, M. P., & Krueger, R. B. (2011b). Response to Halpern's critique of Hypersexual Disorder. *Archives of Sexual Behavior*, 40, DOI: 10.1007/s10508-10011-19753-10501.
17. Krueger, R.B. Critical appraisals of the proposed DSM-5 paraphilia diagnoses. (2011) *Journal of the American Academy of Psychiatry and the Law*, 39: 237. DOI: 10.1080/10720162.2013.775633

2013

18. Carpenter, B.N., Krueger, R.B. Comment on Moser's "Hypersexual disorder: searching for clarity" *Sexual Addiction & Compulsivity*, 20: 59-62

2016

19. Krueger, R.B. Commentary: Diagnosis of Hypersexual or Compulsive Sexual Behavior Can Be Made using ICD-10 and DSM-5 Despite Rejection of This Diagnosis by the American Psychiatric Association. (2016). *Addiction*. 111: 2107-2114

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20. Kraus, S.W., Krueger, R.B., Briken, P. First, M.B., Stein, D.J., Kaplan, M.S., Voon, V., Abdo, Carmita, H.N., Grant, J.E., Atalla, E., Reed, G. M.: Compulsive sexual behaviour disorder in ICD-11 (2018). *World Psychiatry* 17:1, February 2018, 109-110
21. Briken, P., Krueger, R.B. From atypical sexual interests to paraphilic disorders: The planned ICD revisions related to paraphilic disorders. *Journal of Sexual Medicine*. In press.

MONOGRAPHS AND TREATMENT MANUALS

2011

1. Kaplan, MS, Krueger, RB, Vince, M. A Manual for Understanding and Treating Adolescents Who Commit Sexual Offenses. Unpublished manual developed at the Sexual Behavior Clinic, New York State Psychiatric Institute, New York, New York, April 2011

PRESENTATIONS AT PROFESSIONAL MEETINGS (PARTIAL LIST)

1996

1. Krueger RB. The Possible Role of Neuropsychological, Neurological and Neuropsychiatric Examination in the Criminal Process. The New York Head Injury Study Group, New York, New York, June 20, 1996.
2. Krueger RB, Kaplan M. The Assessment and Treatment of Sexual Offenders. Grand Rounds, Kirby Forensic Psychiatric Center, New York, New York, October 10, 1996.
3. Krueger RB, Kaplan M. The Professional Before the Review Board Evaluation, Treatment and Rehabilitation. 1996 Association for the

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Treatment of Sexual Abusers, Annual Convention, Chicago, Illinois, November 12, 1996.

1997

3. Krueger RB, Kaplan M. The Assessment and Treatment of the Adolescent Sexual Offender. Presentation for the Medical Staff of Western New York Children's Psychiatric Center, Buffalo, New York. October 3, 1997.

1998

4. Krueger RB. The Assessment and Treatment of Sexual Dysfunction. Presentation for the Medical Staff of the Family Medicine Program of St. Mary's Hospital, Hoboken, New Jersey, May 26, 1998.
5. Kelly JR, Glancy GD, Krueger RB, Wasyliew OE, Bradford JM. Forensic Evaluation of Paraphilic Stalkers. Presentation at the American Academy of Psychiatry and the Law's Annual Meeting, New Orleans, Louisiana. October 24, 1998.
6. Krueger RB. Management of Pedophilia. Presentation at Grand Rounds, St. Lawrence Psychiatric Center, Ogdensburg, New York, November 11, 1998.

1999

7. Abel G, Krueger RB. Physician Sexual Misconduct. Presentation before the Board of the Committee for Physician's Health, Lakeville, New York. May 7, 1999.
8. Krueger RB. Management of Pedophilia. Presentation before the staff of Buffalo Psychiatric Center, Buffalo, New York, July 15, 1999.
9. Krueger RB, Bradford JM, Glancy GD, Oldham JM, Tucker DE, Maskel L, Lacoursiere R, Ouligian J. The Impact of Sexual Predator Legislation of Various States. Presentation at the American Academy of Psychiatry and the Law's Annual Meeting, Baltimore, Maryland, October 14, 1999.
10. Krueger RB, Abel G, Bradford J, Greenberg DM, Candilis PJ, Berlin FS. The Assessment and Treatment of Child Molesters. Presentation at the American Academy of Psychiatry and the Law's Annual Meeting, Baltimore Maryland, October 15, 1999.
11. Krueger RB. Pharmacological Treatment of Male Sex Offenders. Presentation at the Psychopharmacology Training Program of the New York State Office of Mental Health, Albany, New York, November 9, 1999.

12. Kaplan MS, Krueger RB. A New York State Historical Perspective. Presentation at the Sidney Albert Institute of Parsons Child and Family Center and the New York State Office of Mental Health Conference entitled An Unveiling of an Integrated Statewide System of Care for Child and Adolescent Sexual Abusers. Sagamore, New York, November 10, 1999.
13. Krueger RB, Kaplan MS. Evaluating Sex Offenders. Workshop presentation at the Twelfth Annual NYS OMH Research Conference. Albany, New York, December 7, 1999.

2000

14. Kaplan MS, Krueger RB. Adolescent Sex Offenders An Update. Grand rounds presentation at Rockland Children's Psychiatric Center, Rockland, New York, February 1, 2000.
15. Krueger RB, Kaplan MS. Behavioral and Pharmacologic Treatment of Male Sex Offenders. Statewide live televised grand rounds, New York Office of Mental Health, Bureau of Psychiatric Services, Albany, New York, April 26, 2000.
16. Krueger RB. Evaluation and Treatment of Sex Offenders. Grand rounds presentation at Pilgrim Psychiatric Center, West Brentwood, New York, May 19, 2000.
17. Krueger RB, Kaplan MS. Paraphilias and Professional Sexual Misconduct Evaluation and Treatment. Presentation to the Committee for Physicians' Health of the New York Medical Society, Albany, New York. July 12, 2000.
18. Krueger RB. The Psychopharmacological Treatment of The Paraphilias. Grand rounds presentation at Graystone Park Psychiatric Hospital, Morris Plains, NJ, August 17, 2000.
19. Krueger RB, Kaplan MS. Offender Dynamics. Presentation for Columbia University's Sexual Misconduct Policy Hearing Panelist Training, New York, NY, October 13, 2000.
20. Krueger RB, Hanson KR, Greenberg D. Using research to Improved Risk Assessments for Sex Offenders-Presentation of the Sex Offenders and Psychopharmacology Committees (Advanced). Course at the October 2000 annual meeting of the American Academy of Psychiatry and the Law, Vancouver, British Columbia, Canada. October 20, 2000.
21. Krueger RB, Gratzer T, Brown P, Candilis P, Bradford JM, Smith C. The Ethics of Treatment and Research Involving Antiandrogen Medications-Presentation of the Sex Offenders and Psychopharmacology

Committees. Panel at the October 2000 annual meeting of the American Academy of Psychiatry and the Law, Vancouver, British Columbia, Canada, October 21, 2000.

22. Krueger RB, Hanson KR, Glancy GD, Mossman D, Hart S. Risk Assessment of Sexual Offenders A Critical Assessment--Presentation of the Sex Offenders and Psychopharmacology Committees. Panel at the October 2000 annual meeting of the American Academy of Psychiatry and the Law, Vancouver, British Columbia, Canada, October 21, 2000.
23. Abel GG, Osborn CA, Krueger RB. The Etiology, Evaluation and Treatment of Professionals Accused of Sexual Misconduct with Their Clients. Session at the November 2000 annual meeting of the Association for the Treatment of Sexual Abusers. San Diego, California, November 3, 2000.
24. Krueger RB. The Pharmacological Treatment of the Paraphilias. Presentation at Teaching Conference at Bronx State Psychiatric Center, Bronx, NY, November 10, 2000.
25. Krueger RB. The Pharmacological and Behavioral Treatment of the Paraphilias. Presentation at Grand Rounds, Hutchings Psychiatric Center, Syracuse, NY, November 15, 2000.
26. Krueger RB. An Update on Current Evaluation and Treatment of the Paraphilias. Presentation at Grand Rounds, Queens Hospital Center, Department of Psychiatry, Jamaica, NY, December 15, 2000.

2001

27. Krueger RB. Recent Developments in the Treatment of the Paraphilias and the Hypersexual Disorders. Presentation at Grand Rounds, New York Presbyterian Hospital, Cornell University, Payne Whitney Clinic, New York, NY, March 7, 2001.
28. Krueger RB. Recent Developments in the Treatment of the Paraphilias and the Hypersexual Disorders. Presentation at Grand Rounds, New York Presbyterian Hospital, Westchester Division, White Plains, NY, April 24, 2001.
29. Krueger RB, Kaplan, MS. Polygraphy and Sex Offenders. Presentation at the Tristate Meeting of the U.S. Department of Probation, Southern District, Manhattan, NY, May 11, 2001.
30. Krueger RB. Recent Developments in the Treatment of the Paraphilias and the Hypersexual Disorders. Presentation at Grand Rounds, Kingsborough Psychiatric Center, Brooklyn, NY, May 16, 2001.

31. Krueger RB. Recent Developments in the Treatment of the Paraphilias and the Hypersexual Disorders. Presentation at Grand Rounds, Coney Island Hospital, Brooklyn, NY, June 4, 2001.
32. Krueger RB. Recent Developments in the Treatment of the Paraphilias and the Hypersexual Disorders. Presentation at Grand Rounds, Rockland Psychiatric Center, Orangeburg, NY, October 4, 2001.
33. Krueger RB. Recent Developments in the Treatment of the Paraphilias and the Hypersexual Disorders. Presentation at Grand Rounds, Creedmore Psychiatric Center, Queens, NY, October 5, 2001.
34. Krueger RB, Abel G, Kuban M, Bradford J, Sosnowski D. Objective Methods for Assessing Sexual Interest: A Critical Appraisal. Presentation at the Annual Meeting of the American Academy of Psychiatry and the Law, Boston, MA, October 27, 2001.
35. Krueger RB, Kafka M. Male Hypersexuality Disorders New Clinical Insights and Pharmacological Treatments. Presentation at the Annual Meeting of the American Academy of Psychiatry and the Law, Boston, MA, October 27, 2001.
36. Glancy G, Brown P, Krueger RB, Ouligian J, Candilis P. Psychopharmacology of Violence. Presentation at the Annual Meeting of the American Academy of Psychiatry and the Law, Boston, MA, October 28, 2001.

2002

37. Krueger RB. Recent Developments in the Treatment of the Paraphilias and the Hypersexual Disorders. Presentation at Grand Rounds, Mohawk Valley Psychiatric Center, Utica, New York, March 7, 2002.
38. Krueger RB. Risk Assessment and Treatment of a Patient with a History of Multiple Homicides. Presentation at Grand Rounds, St. Lawrence Psychiatric Center, Ogdensburg, New York, April 3, 2002.
39. Krueger RB. Recent Developments in the Treatment of the Paraphilias and the Hypersexual Disorders. Presentation at Grand Rounds, Harlem Hospital, Harlem, New York, April 17, 2002.
40. Krueger RB. The Use of Medication in the Treatment of Sex Offenders. Presentation at the Seventh Annual Training Conference on the Evaluation and Treatment of Sex Offenders Poughkeepsie, NY, April 30, 2002.
41. Krueger RB. The Use of Medication in the Treatment of Individuals with Paraphilias and Hypersexual Disorders: An Update. Presentation at

Grand Rounds, the Department of Psychiatry, Montefiore Medical Center and the Albert Einstein College of Medicine, Bronx, NY, October 17, 2002.

42. Krueger RB. Recent Developments in the Diagnosis and Pharmacological Treatment of Sex Offenders. Presentation at Grand Rounds, Kirby Forensic Psychiatric Center, Ward's Island, New York, NY, November 7, 2002.

2003

43. Krueger RB. Recent Developments in the Diagnosis and Pharmacological Treatment of Sex Offenders. Presentation at Grand Rounds, Middletown Psychiatry Center, Middletown, NY, February 5, 2003.
44. Kaplan MS, Krueger RB. Assessment of Youngsters with Histories of Inappropriate Sexual Behavior. Presentation at Grand Rounds, Rockland Children's Psychiatric Center, Orangeburg, NY, February 11, 2003.
45. Krueger RB. Recent Developments in the Diagnosis and Pharmacological Treatment of Sex Offenders. Presentation at Mount Sinai Hospital, New York, NY, March 11, 2003.
46. Krueger RB. Recent Developments in the Treatment of the Paraphilias and the Hypersexual Disorders in Adults and Adolescents. Presentation at Combined Social Work and Psychiatry Grand Rounds, Harlem Hospital, New York, NY, March 26, 2003.
47. Krueger RB. Recent Developments in the Diagnosis and Pharmacological Treatment of Sex Offenders. Presentation at Grand Rounds, Manhattan Psychiatric Center, Ward's Island, New York, NY, May 2, 2003.
48. Krueger RB. Recent Developments in the Diagnosis and Pharmacological Treatment of Sex Offenders. Presentation at Grand Rounds, Pilgrim Psychiatric Center, West Brentwood, NY, November 5, 2003.

2004

49. Krueger RB. Recent Developments in the Diagnosis and Pharmacological Treatment of Sex Offenders. Presentation at Grand Rounds, St. Luke's-Roosevelt Hospital Center, New York, NY, March 10, 2004.

2005

50. Kaplan MS, Krueger RB. Assessment and Treatment of Adolescent Sexual Offenders. Presentation at Grand Rounds, Rockland Children's Psychiatric Center, Orangeburg, NY, February 1, 2005.
51. Krueger RB, Kaplan MS. Assessment and Treatment of Individuals Arrested for Internet Crimes Against Children. Presentation at the Tenth Annual Conference of the New York State Alliance of Sex Offender Service Providers and the New York State Chapter of the Association for the Treatment of Sexual Abusers. White Plains, NY, May 6, 2005.
52. Rotter M, Reid N, Rivera J, McCullough M, Krueger R. Psychopharmacological Assessment and Treatment of Patients in the New York State OMH System. Part of an overall presentation The Mentally Ill Sexual Offender Special Considerations for a Special Population. Presentation at the Tenth Annual Conference of the New York State Alliance of Sex Offender Service Providers and the New York State Chapter of the Association for the Treatment of Sexual Abusers. White Plains, NY, May 6, 2005.
53. Freedman PM, Kaplan MS, Krueger RB, Simring SS. Sexual Deviation, Sexual Crimes, and Sexually Violent Predators. State of New Jersey, Judicial College, Administrative Office of the Courts and the Supreme Court Committee on Judicial Education. Teaneck, NJ. November 22, 2005.

2006

54. Krueger RB. Recent Developments in the Diagnosis and Pharmacological Treatment of Sex Offenders. Presentation at Grand Rounds, Creedmore Psychiatric Center, New York, NY, March 9, 2006.
55. Krueger RB. Recent Developments in the Diagnosis and Pharmacological Treatment of Sex Offenders. Presentation at Grand Rounds, Gouvernor Hospital, New York, New York, April 25, 2006.
56. Krueger, RB. Reports of Child Sexual Abuse Perpetrated by Females in a Sample of Male Sexual Offenders and Comments on SVP Legislation in New York State. Presentation at a conference "Meeting the Needs of All Victims of Criminal Behavior" given by New York AAPL, New York University, New York, New York, April 29, 2006.
57. Friedman, RA, Kaplan, MS, Weiss, SL, Greenberg, M. Risk Assessment and a Team Approach to Sexual Violence in the Family. Presentation at The Interdisciplinary Forum on Mental Health and Family Law in New York State. New York, New York, May 16, 2006.
58. Abel, GG, Marks, TI, Krueger, RB. The Evaluation and Treatment of Physicians Involved in Sexual Misconduct and Sexual Harassment.

Course 100. The American Psychiatric Association's 159th Annual Meeting. Toronto, Ontario, Canada, May 25, 2006.

59. Krueger, RB, Kaplan, MS. Biological Treatments for the Paraphilias and Compulsive Sexual Behavior. Presentation at Annual Meeting of the Association for the Treatment of Sexual Abusers. Chicago, Illinois, September 29, 2006.
60. Rotter, M, Krueger, R. The Mentally Ill Sexual Offender: Special Considerations for a Special Population. Presentation by Dr. Rotter before the American Psychiatric Association's Institute on Psychiatric Services Annual Meeting. New York, New York, October 6, 2006.
61. Seybert, J, Hurley, D, Whalen, M, Garoppolo, T, Bode, A, Krueger, R. Selected Issues Involving Child Pornography. Presentation at the Criminal Justice Act Panel Committee of the Eastern District of New York. Brooklyn, New York, October 24, 2006.
62. Corliss, RK, Cross, CD, Krueger, RB, Harkavy, S, O'Connor, A: Civil Commitment of Sexually Violent Predators: Where Are We Now and Where Do We Want to Go? NAMI-New York State 24th Annual Educational Conference. From Research to Recovery: Improving the Lives of New Yorkers with Mental Illness. White Plains, New York, November 3, 2006.
63. Kinscherff, R, Guidry, L, Berlin, F, Bengis, SM, Hart, SD, Latham, C, Prentky, R, Vincent, G, Dunseith, NW, Taylor, P, Krueger, RB, Saleh, FM, Ball, CJ. Conversations on Best Practice Applications for the Mentally Ill/ Problematic Sexual Behavior Population: Values and Implementation. Worcester, Massachusetts, November 17, 2006.
64. Krueger, RB. Influencing sex offender legislation: What role for Clinicians? Invited lecture for a symposium entitled: Society and sex offenders with D, Wylie, K, Morris, C, Green, P, Krueger, R, Bradford, J, and Soothill, K. Organized by the Sexuality & Sexual Health Section of the Royal Society of Medicine, London, England, December 1, 2006.
65. Krueger, RB. Biological Treatments for the Paraphilias and Compulsive Sexual Behavior. Presentation at Grand Rounds. Central New York Psychiatric Center, Marcy, New York, December 20, 2006.
66. Krueger, RB. Biological Treatments for the Paraphilias and Compulsive Sexual Behavior. Presentation at Grand Rounds. Woodhull Medical & Mental Health Center, January 17, 2006.

67. Baillargeon, J. et al. Responses to Sex Crimes and People Convicted of Sex Crimes. Participant. Open Society Institute, New York, New York, January 25, 2006.

2007

68. Abel, GG, Marks, TI, Krueger RB. The Evaluation and Treatment of Physicians Involved in Sexual Misconduct and/or Sexual Harassment. American Psychiatric Association 160th Annual Meeting, San Diego, California. May 21, 2007.
69. Lindenmeyer, JB, Langer, SJ, Krueger, RB, Barbaree, Barbaree, HE, Siegel, LA, Zonana, HV. Biological Treatments for the Sexually Violent Offender and Discussant in Symposium 70. The Sexually Violent Offender. American Psychiatric Association 160th Annual Meeting, San Diego, California. May 23, 2007.
70. Krueger, RB. Risk Assessment and Risk Management. Presentation at the Whiting Forensic Division of Connecticut Valley Hospital, Middletown, Connecticut. September 20th, 2007.
71. Krueger, RB. Paraphilias & Hypersexual Disorders: An Update. Presentation at Grand Rounds, Department of Psychiatry, Mount Sinai Hospital, New York, New York. September 25th, 2007.
72. Seybert, J, Krueger, R, Kelly, E., Goldberg, A, Whalen, M, Levitt, R. Psychiatric/Mental Health Involvement in Cases Involving Sexual Crimes. Part of a panel entitled "Rita, Rita, Guidelines Eater and SORNA, SMART, AWA, and SOMA by the Criminal Justice Act Panel Committee of the Eastern District of New York, Brooklyn, New York. November 7th, 2007.
74. Krueger, R, Bradford, J. Compulsive Sexual Behavior, Discussant. New York State Office of Mental Health, Bureau of Psychiatric Services and Research Institute Support, Psychopharmacology Training Program, Albany New York, November 29th, 2007.

2008

75. Miraglia, R, Krueger, R, Kaplan, M. The Treatment and Management of Persons Who Have Committed Sexual Offenses, Bureau of Psychiatric Services and Research Institute Support, Statewide Grand Rounds, Albany, New York, February 27th, 2008.
76. Krueger, R, Paraphilias & Hypersexual Disorders: An Update. Presentation at Grand Rounds, Mid-Hudson Forensic Psychiatric Center, New Hampton, New York. April 22nd, 2008.

77. Krueger, R, Kaplan, M. The Diagnosis and Treatment of Individuals Convicted of Internet Crimes Against Children. Workshop at Annual Conference of the New York State Association for the Treatment of Sexual Abusers, Saratoga Springs, New York. May 13th, 2008.
78. Krueger, R, Kaplan, M. Medication Treatment of Paraphilias and Compulsive Sexual Behavior. Workshop at Annual Conference of the New York State Association for the Treatment of Sexual Abusers, Saratoga Springs, New York. May 13th, 2008.
79. Miraglia, R, Culkin, J, Lee, G, Krueger, Richard. Current Trends in Management and Treatment of Sex Offenders: Psychopharmacological Approaches. Grand Rounds. Bronx State Psychiatric Center. Bronx, New York September 5th, 2008.
80. Nersessian, E, Levy, F, Davidson, A, Kernberg, O, Krueger, R, Williams, L, Winemaker. Paraphilias. A Roundtable Discussion. The Philoctetes Center for the Multidisciplinary Study of Imagination at the New York Psychoanalytic Institute, New York, New York. October 18th, 2008.
81. Weiss, S, Braunstein, L, Kaplan, M, Krueger, R. Internet pornography and child custody. Presentation at The Interdisciplinary Forum on Mental Health and the Law, New York, New York. December 18th, 2008.

2009

82. Zucker, K, Blanchard, R, Kafka, M, Krueger, R, Meyer-Bahlburg, H, Seagraves, R.T., Brotto, LA, Blinik, I. Symposium: Provisional report by the DSM-V workgroup on sexual and gender identity disorders. Presentation at the 34th Annual Meeting of the Society for Sex Therapy and Research, Arlington, Virginia, April 3rd, 2009.
83. Krueger, R, Lee, L. Current Trends in Management and Treatment of Sex Offenders: Psychopharmacological Approaches. Central New York Psychiatric Center, Utica, New York, April 21st, 2009.
84. Lee, L.G., Krueger, R. Androgen Reduction Therapy in SOTP. Joint CNYPC/LSPC/Central Office Session, Utica, New York, April 21st, 2009.
85. Krueger, R, Kaplan, M. Diagnostic Issues in the Assessment and Treatment of Individuals Convicted of Sexual Offenses. Central New York Psychiatric Center, Utica, New York, April 21st, 2009.
86. Krueger, R., Lee, L.G. Psychopharmacological Treatment of Sexual Offenders. Presentation to Judges, MHLS attorneys, and the Attorney General's Office. Ceremonial Courtroom, Onondaga County Courthouse, Syracuse, New York, June 4th, 2009.

87. Kafka, M, Blanchard, R, Krueger, R, Langstrom, N, Paraphilic Disorders and DSM-V: Considerations for Revision of Diagnostic Criteria; Sexual Sadism Disorder, Sexual Masochism Disorder, and Paraphilic Coercive Disorder. Annual Meeting of the Association for the Treatment of Sexual Abusers, Dallas, Texas, October 1st, 2009.
88. Krueger, R, First, M, Fedoroff, P, Zonana, H, Berlin, F: A Critical Appraisal of Proposed DSM-V Paraphilic Diagnoses. Annual Meeting of the American Academy of Psychiatry and the Law, Baltimore, Maryland, October 29th, 2009.
89. Kaplan, M, Krueger, R: Proposed Paraphilic Diagnoses for DSM-V. Grand Rounds, Mid-Hudson Forensic Psychiatric Center, New Hampton, New York, December 1st, 2009.

2010

90. Krueger, R, Hall, D, Thomas, N, McPherson, J, Page, J, Carey, C. Testimony regarding proposed sex offender legislation and SORA actuarial assessment before the New York State Senate Committee on Crime, Crime Victims, and Corrections. Albany, New York, March 9th, 2010.
91. Krueger, R, Kaplan, M: Paraphilic diagnoses in DSM-V at a conference entitled "Contemporary Treatments of Paraphilias" at the Inbal Hotel in Jerusalem, Israel on June 17th, 2010.
92. Kaplan, M, Krueger, R: Cognitive-behavioral treatment of paraphilias and Hypersexual Disorders at a conference entitled "Contemporary Treatments of Paraphilias" at the Inbal Hotel in Jerusalem, Israel on June 17th, 2010.
93. Krueger, R: Medication treatment of the paraphilias and hypersexual disorders. Grand Rounds at the Ann Klein Forensic Center Special Treatment Unit, Rahway, New Jersey, July 14th, 2010.
94. Krueger, R: New developments in the diagnosis of sexual masochism and sexual sadism. Presentations at the 11th Conference of the International Association for the Treatment of Sexual Offenders (IATSO), Oslo, Norway, September 2nd, 2010.
95. Maskel, L, Bradford, J, Tucker, D, Krueger, R, Thornton, D, Zander, T, Franklin, K: Stirring the DSM-5 Cauldron. Debate at the 41st Annual Meeting of the American Academy of Psychiatry and the Law, Tucson, Arizona, October 23rd, 2010.

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96. Krueger, R: Medication Treatment of the Paraphilias and Hypersexual Disorders. Presentation at Kingsboro Psychiatric Center, Brooklyn, New York, October 28th, 2010.

2011

97. Krueger, R: Biological Treatment of Individuals with Paraphilias and/or Sexual Disorders. Invited lecture for the Faculty of Forensic Psychiatry, Royal College of Psychiatrists, Annual Meeting, Berlin, Germany, February 3rd, 2011.
98. Kaplan, M, Krueger, R: Sexual Acting Out in Adolescents: Normative vs. Pathological Behaviors and Their Management. Presentation at Grand Rounds at Queens Children's Psychiatric Center, Bellerose, New York, October 20th, 2011.
99. Sorrentino, Rene, Kafka, M, Krueger, R: The DSM-5 Sexual Disorders: Forensic Implications. Presentation at the Annual Meeting of the American Academy of Psychiatry and the Law, Boston, Massachusetts, October 24th, 2011.
100. Lee, Li-Wen, Miraglia, R, Krueger, R: New York State's SVP Law: Program Development and Implementation. Presentation at the Annual Meeting of the American Academy of Psychiatry and the Law, Boston, Massachusetts, October 26th, 2011.
101. Krueger, R, Kaplan, M: Treatment of Individuals with Paraphilias and/or Sexual Disorders. Presentation at Grand Rounds. Lincoln Hospital, Bronx, New York, December 20th, 2011.

2012

102. Krueger, R: Paraphilic Diagnoses in DSM-5 and Treatment Update on Paraphilic Disorders. Presentation at Grand Rounds. Department of Psychiatry. Nassau University Medical Center, East Meadow, New York, February 29th, 2012.
103. Krueger, R: Biological Treatment of Individuals with Paraphilias and/or Sexual Disorders. Keynote address to the 12th Annual Massachusetts Department of Mental Health Forensic Service Division's Mentally Ill/Problematic Sexual Behavior Program Training conference. University of Massachusetts Hoagland-Pincus Conference Center, Shrewsbury, Mass. May 17th, 2012.
104. Krueger, R, Lee, Grace: The Psychopharmacological Treatment of Individuals with Paraphilias and/or Sexual Disorders. Workshop given at

the Annual Conference of the New York State Association for the Treatment of Sexual Abusers, Hunter, New York. May 22nd, 2012.

105. Krueger, R, First, M, Kaplan, M: Forensic Implications of DSM-5. Workshop given at the Annual Conference of the New York State Association for the Treatment of Sexual Abusers, Hunter, New York. May 22nd, 2012.
106. Krueger, R. Behavioral Addictions. Conference on the Essentials of Addiction given at the Association of the Bar, City of New York, New York, New York, June 4th, 2012
107. Mossberg, M., Ravitz, A, Tatarsky, A, Drager, JL, Bender, S, Alter, E. The Essentials of Addictions: A New Understanding for Family Law Practitioners, Forensic Experts and the Court. Conference at the Association of the Bar, City of New York, New York, New York, September 27th, 2012
108. Krueger, R: The Role of Psychiatric Diagnosis and Treatment in the Management of Individuals Who Have Committed Sexual Crimes. Invited Lecture for Course on Sex Offenders in the Criminal Justice System arranged by the Honorable Barnett Hoffman at John Jay College, New York, New York, October 16th, 2012
109. Krueger, R: Biological Treatment of Individuals with Paraphilias and/or Sexual Disorders. Invited Presentation at the Special Treatment Unit, Avenel, New Jersey, October 17th, 2012
110. Maskel, L, Forestell, M, Singer, M, LeBell, R, Corcoran, J, Krueger, R, Wollert, R, Bradford, J.: Hebeophilia: Weed Diagnosis in the Botanical Garden of DSM-5? Mock Trial, 43rd Annual Meeting of the American Academy of Psychiatry and the Law, Montreal, Quebec, Canada, October 25th, 2012.

2013

111. Krueger, R, Kaplan, M: Diagnostic Issues in the Assessment and Treatment of Individuals Convicted of Sexual Offenses and an Update on DSM-5. Grand Rounds, Mid-Hudson Forensic Psychiatric Center, New Hampton, New York, February 5th, 2013
112. Krueger, R: Sex Offender Diagnoses, Treatment, and DSM-5 Update. The National Alliance of Sentencing and Mitigation Specialists (NASAMS), Baltimore, Maryland, March 23rd 2013
113. Krueger, R: Pain and Sadomasochism: Definitions, Epidemiology, Studies, Mechanisms, and Future Directions. Invited Presentation to The

Pain Project, University of Glasgow, Department of Philosophy, Glasgow, Scotland, April 6th, 2013

114. Krueger, R: Paraphilic Disorders: Presentation at Meeting on Protocol Development for Field Testing of ICD-11 Sexual Disorders and Sexuality-Related Conditions. Invited presentation and participation in meeting hosted by The World Health Organization for the development of ICD-11. Geneva, Switzerland, April 23rd-25th, 2013
115. Krueger, R: Paraphilic Disorders in ICD-11. Presentation at the National Transgender Health Summit, San Francisco, California, May 18th, 2013
116. Krueger, R: Sexual Disorders and Sexual Health in the ICD-11: Parallels and Contrasts with DSM-5: Paraphilic Disorders, with Dresher, J, Cohen-Kettenis, Peggy, Cochran, Susan, Giami, Alain, and Reed, Geoffrey. Symposium at the American Psychiatric Association's Annual Meeting, San Francisco, California, May 21st, 2013
117. Krueger, R: Pain and Sadomasochism: Definitions, Epidemiology, Studies, Mechanisms, and Future Directions. Invited Presentation to The Pain Project, University of Glasgow, Department of Philosophy, Glasgow, Scotland, June 19th, 2013
118. Krueger, R, Kaplan, M.: Sex Offender Diagnosis, Treatment, and DSM-5 Updates. Multi-Track Federal Criminal Defense Seminar, Buffalo, New York, August 9th, 2013
119. Krueger, R: Biological Treatment of Individuals with Paraphilias and Hypersexual Disorders. Grand Rounds, Department of Psychiatry, Jacobi Hospital, Bronx, New York, September 25th, 2013
120. Krueger, R: The Role of Psychiatric Diagnosis and Treatment in the Management of Individuals Who Have Committed Sexual Crimes. Invited Lecture for Course on Sex Offenders in the Criminal Justice System arranged by the Honorable Barnett Hoffman at John Jay College, New York, New York, October 8th, 2013
121. Krueger, R: DSM-5 For Article 10 Examiners. Presentation at the New York State Office of Mental Health Forensic Division, Albany, New York, October 18th, 2013
122. Krueger, R, Kleinplatz, PJ, Silverstein, C: Kinky Sex of Paraphilia? Sexual Disorders in DSM-5, Annual Meeting of the New York State Psychological Association, New York, New York, October 20th, 2013
123. Lee, Li-Wen, Krueger, Richard, Murphy, Lisa, Knack, Natasha, Maskel, Lynn: Ethical Issues in SVP Treatment: Is There a Right Answer?

Presentation at the Annual Meeting of the American Academy of Psychiatry and the Law, San Diego, California, October 25th, 2013

124. Krueger, Richard, Martinez, Nicolas: The Classification of Sexual Disorders and Sexual Health: Recommendations for ICD-11 and Legal and Policy Issues Related to Proposals for Paraphilic Disorders in WHO's iCD-11. Presentation at the Annual Meeting of the American Academy of Psychiatry and the Law, San Diego, California, October 25th, 2013
125. Kamdang, L, Guevara, V., Krueger, R: Discussion on Child Pornography and People in the Registry. Discussion with the Policy Caucus and Criminal Justice Caucus, Columbia University School of Social Work, New York, New York, November 13th, 2013
126. Krueger, R, Kaplan, M: The Case of an Individual Arrested for Child Pornography: The U.S. vs. C.R. Presentation at the Tri-State American Academy of Psychiatry and the Law's 15th Annual Conference, San Juan, Puerto Rico, December 13th, 2013

2014

127. Krueger, R: DSM-5 and Sexual Disorders. Grand Rounds. Mid-Hudson Forensic Psychiatric Hospital, New Hampton, New York, April 15th, 2014
128. Krueger, R: Proposed Diagnostic Criteria and Biological Therapies for Compulsive Sexual Behavior Disorder: DSM-5 and ICD-11 Considerations in Symposium by Potenza, M, Erez, G, Levounis, P, Kraus, S, Kafka, M. Hypersexual Behaviors: Diagnostic, Categorical, Gender, Orientation, Psychopathological, and Treatment Considerations. The Annual Meeting of the American Psychiatric Association, New York, New York, May 5th, 2014
129. Krueger, R: Sexual Health and Sexual Disorders: The Science and Implications of Changing Diagnoses in the ICD. Presentation for the Columbia University Seminar Series on Global Mental Health, Columbia Mailman School of Public Health, New York, New York, May 12th, 2014
130. Krueger, R., Reed, G., First, M., Briken, P.: Proposed Paraphilic Disorders for ICD-11. Poster Presentation at the Fortieth Annual Meeting of the International Academy of Sex Research, Dubrovnik, Croatia, June 26th, 2014
131. Briken, P., Krueger, R.: Paraphilic Diagnoses in DSM-5 and Proposals for ICD-11—Are There Implications for Sexual Offender Management? Presentation at the 13th Conference of the International Association for the Treatment of Sexual Offenders (IATSO), Porto, Portugal, September 3rd, 2017

132. Turner, D., Krueger, R., Harrison, K, Briken, P.: Institutional and Legal Prerequisites of Pharmacological Sexual Offender Treatment in the Countries of the European Union and North American. Presentation at the 13th Conference of the International Association for the Treatment of Sexual Offenders (IATSO), Porto, Portugal, September 4th, 2017

2015

133. Krueger, R.: Biological Treatment of Individuals with Paraphilias and Hypersexual Disorders. Presentation at the University of Minnesota's Department of Psychiatry and Sexual Health Grand Rounds, Minneapolis, Minnesota, February 18th, 2015
134. Krueger, R.: Paraphilic Disorders in DSM-5. Presentation at the 40th Annual Conference of the Forensic Mental Health Association of California, Monterey, California, March 19th, 2015
135. Krueger, R.: Paraphilic Disorders in ICD-11. Presentation at the 40th Annual Conference of the Forensic Mental Health Association of California, Monterey, California, March 19th, 2015
134. Krueger, R.: The Role of Androgen Reduction Therapy for the Treatment of Individuals Who Have Committed Sexual Offenses. Presentation at the 40th Annual Conference of the Forensic Mental Health Association of California, Monterey, California, March 19th, 2015
136. Krueger, R., Reed, G., First, M., Briken, P., Marias, A, Kismodi, E.: Proposed Paraphilic Disorders for ICD-11. Poster Presentation at the Forty-first Annual Meeting of the International Academy of Sex Research, Toronto, Canada, August 11th, 2015
137. Krueger, R., Maskel, L., Bradford, J., Zonana, H., Dwyer, G., Wollert, R.: Between a Rock and a Hard Place: Debating Sexual Sadism Diagnoses. Panel at the 46th Annual Meeting of the American Academy of Psychiatry and the Law, Fort Lauderdale, Florida, October 25th, 2015
138. Krueger, R., Murphy, L, Dwyer, G, Knack, N: Child Pornography Offenders: Definitions, assessment, risk of recidivism, and treatment. Panel at the 46th Annual Meeting of the American Academy of Psychiatry and the Law, Fort Lauderdale, Florida, October 23rd, 2015

2016

139. Krueger, R, Kaplan, M, Weiss, S, Braunstein, L.: The Impact of Sexual Behavior on Custody Evaluations. Presentation at the Interdisciplinary

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Forum on Mental Health and Family Law, Phillips-Nizer, New York, New York, April 14th, 2016

140. Krueger, R, Kaplan, M, Menzie, L, Rzewinski, J, Kaplan, M, Jackson, K, Ford, W.: Program Descriptions on Sex Offender Treatment Panel of Providers. Presentation before the New York County District Attorney's Office, May 4th, 2016, New York, New York
141. Krueger, R: Psychopharmacology for Forensic Psychologists. Lecture at Fairleigh Dickenson University, Teaneck, New Jersey June 15th, 2016
142. Krueger, R, Briken, P: Proposed Paraphilic Disorders and Impulse Control Disorders for the International Classification of Disease and Related Health Problems Eleventh Revision (ICD-11). Presentation at the 14th Biannual Conference of the International Association for the Treatment of Sexual Offenders, Copenhagen, Denmark, September 8th, 2016
143. Murphy, L, Dwyer, G, Krueger, R, Fedoroff, P: The Scarlet Letter: Public Notification of Sexual Offenders. Presentation at the Annual Meeting of the American Academy of Psychiatry and the Law, Portland Oregon, October 30th, 2016
144. Krueger, R, Kaplan, M. Sexual Masochism and Non-Consenting Victim Case Presentation. Presentation at the Annual Winter Conference of Tri-State AAPL, Miami, Florida, December 8th, 2016

2017

145. Krueger, R. Current Practices for Treating Sexual Offenders. Presentation at a joint meeting of the Council on Psychiatry and the Law and the Committee for Judicial Action of the American Psychiatric Association. Washington, D.C. September 15th, 2017
146. Krueger, R., Ser, Jason: Sex Offender Risk Assessment. Presentation at a conference on Sentencing Strategy Session for Federal Public and Community Defenders, Chicago, Illinois, September 26th, 2017
147. Krueger, R. Paraphilic Disorders in ICD-11. Presentation at the World Congress of Psychiatry, Berlin, German, October 10th, 2017
148. Candilis, P, Kaempf, A, Kolla, N, Krueger, R. Truth to Power: Committee Chairs and the Presidential Theme Ethics Committee. Truth to Power in the Matter of Sexual Offenders. American Academy of Psychiatry and the Law. Annual Meeting, Denver, Colorado, October 26th, 2017
149. Vince, M, Kaplan, M, Krueger, R. The Adolescent Sexual Offender Program at New York State Psychiatric Institute. Presentation to the

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New York County Department of Probation, New York, New York,
November 8th, 2017

150. Krueger, R, Kaplan, M. The Diagnosis and Treatment of Individuals Who Have Committed Sexual Offenses-Part I-Evaluation. Presentation at Grand Rounds to the Mid-Hudson Forensic Psychiatric Center, New Hampton, New York, November 14th, 2017
151. Krueger, R, Kaplan, M. The Diagnosis and Treatment of Individuals Who Have Committed Sexual Offenses-Part II-Treatment. Presentation at Grand Rounds to the Mid-Hudson Forensic Psychiatric Center, New Hampton, New York, December 19th, 2017

2018

152. Kaplan, M, Krueger, R. The Sexual Behavior Clinic at New York State Psychiatric Institute. Presentation to Behavioral Health Services, Adult and Juvenile Operations, NYC Department of Probation, New York, New York, January 17th, 2018
153. Kaplan, M, Krueger, R. Child Pornography Offenders. Presentation to the Manhattan Sex Therapy Group. New York, New York April 10th, 2018
154. Drescher, J, Reed, G., Krueger, R, Kraus, S, Reshuffling the Deck: Revising Sexual and Gender Diagnoses in ICD-11. Accepted presentation. Annual Meeting of the American Psychiatric association, New York, New York, May 7th, 2018

EXHIBIT B4



Richard B. Dauber, Ph.D. Lic. No. 1688	Jayne Schachter Walco, Ph.D. Lic. No. 3194
Kenneth Freundlich, Ph.D. Lic. No. 2122	Nathan McClelland, Ph.D. Lic. No. 5510
Daniel N. Watter, Ed.D., FAACS Lic. No. 2615	Hayley Hirschmann, Ph.D. Lic. No. 3802
Francine Rosenberg, Psy.D. Lic. No. 3694	Carly Orenstein, Psy.D. Lic. No. 4441
Daniel DaSilva, Ph.D. Lic. No. 3720	Toby G. Dauber, L.C.S.W. Lic. No. 08200
Stuart Leeds, Psy.D. Lic. No. 3863	Ashley Gorman, Ph.D., ABPP Lic. No. 4721
Jeffrey C. Singer, Ph.D. Lic. No. 3444 NY Lic. No. 12999	Larisa R. Wainer, Psy. D. Lic. No. 5033
Josh Glawe, L.C.S.W. Lic. No. 53407	Joanne Van Nest, Ph.D. Lic. No. 3183
	Lisa G. Hahn, Ph.D., ABPP Lic. No. 4625

March 7, 2019

Robert A. Bianchi, Esq.
Bianchi Law Group, LLC
4 York Avenue, Second Floor
West Caldwell, New Jersey 07006

RE: S. Bradley Mell

Dear Mr. Bianchi,

As you know, I have been treating Mr. S. Bradley Mell since August 2018. Since that time, I have met with Mr. Mell for 22 psychotherapy sessions. I am pleased to report that Mr. Mell has been an extremely cooperative and compliant psychotherapy patient. Mr. Mell appears to have taken the psychotherapy process seriously, and seems to be highly motivated to better understand his aberrant behavior, and ensure that it is not repeated in the future.

Since you are well versed in the particulars of Mr. Mell's arrest for inappropriate sexual behavior, I will desist from providing such details. Suffice to say that Mr. Mell found himself in a great deal of legal difficulty given his sexual affair. His prior therapist, Dorothy O'Keefe Diana, LCSW, referred Mr. Mell to me. Ms. Diana was aware that my practice specializes in the assessment and treatment of sexual disorders, and she believed that I would be better able to assist Mr. Mell than she. By way of introduction, allow me to describe some of my background for such work. I received my doctorate from New York University in 1985 and have been providing sex therapy evaluations and treatments since that time. I am a licensed psychologist in the State of New Jersey, and am also a board-certified sex therapist (AASECT & ABS). In 1995 I became a clinical member of the Association for the Treatment of Sexual Abusers (ATSA), and for the past several years I have served on the Special Classification Review Board (SCRB) of the Adult Diagnostic and Treatment Center (ADTC), New Jersey's sex offender prison. I am also a consultant to the Physician Assistance Program of New Jersey (PAP-NJ). In addition, I am a current member of the State of New Jersey Board of Psychological Examiners, having been appointed by both Governors Corzine and Christie. A



published author and frequent lecturer/training for sexuality professionals, I am also the Immediate Past-President of the Society for Sex Therapy and Research (SSTAR). I have attached my complete C.V. to this letter for your perusal.

After spending several months working with Mr. Mell, I am of the opinion that he is not a sex offender, per se. True, his actions resulted in a sexual offense, but his behavior was not motivated by the cravings of a sexual predator. Rather, I see Mr. Mell as a trauma victim, and his actions are those of a man who was triggered by traumatic events from his distant and recent past. Over the past many years, I have worked with dozens of men (and some women) whose sexual behavior became uncharacteristically aberrant and uncontrolled as a reaction to a confrontation with mortality and death. According to Mr. Mell and the records provided to me, Mr. Mell has had no history of aberrant sexual behavior, including extra-marital affairs, until his problematic sexual relationships in 2015. According to the history, in 2011 Mr. Mell lost two close friends in the crash of his private helicopter. The friends were coming home from a golf trip, and Mr. Mell was supposed to be on that trip instead of one of his friends. At the last minute, business obligations interfered with Mr. Mell's being able to go on the trip, and he offered his spot to a friend. Mr. Mell reports that he was never the same after the death of his friends, and suffered with a combination of survivor's guilt and extreme anxiety regarding his own mortality and how close he himself came to death. Mr. Mell entered therapy at this time with another therapist, but his death-terror did not abate. Lawsuits followed and finally resolved in May 2016. Mr. Mell's first affair began 3 months later. This continued for approximately one year, and ended when the girl announced she had a new boyfriend. This led to the second affair with L.C.

In addition to the tragedy of the helicopter crash, Mr. Mell has experienced a lifetime of trauma. His parents divorced when he was in 9th grade due to his father's extramarital affair with his mother's best friend. He recalls a generally chaotic home characterized by tension and dysfunction, as well as having to testify in his parent's divorce trial against his father. His mother began heavy use of benzodiazepines, and he would often find her passed out at home upon his return from school. His father was an alcoholic and committed suicide. Mr. Mell recalls a highly ambivalent relationship with his father, and vowed to never be like him. Of note, however, is that Mr. Mell began his extramarital affairs when he was the same age as his father was when beginning his extramarital exploits. There are other behaviors that mirror his father's, but none of them manifested until after the helicopter crash and death of friends. Mr. Mell struggled in school as a youth, and teachers suggested he might have been mildly mentally retarded. Later on, he was correctly diagnosed with dyslexia, and was able to succeed in school beyond the expectations of all. In 2003, Mr. Mell was diagnosed with a life-threatening illness of the bile duct, and was told he could expect a life expectancy of another 8-10 years. That was 16 years ago, and Mr. Mell reports living for 10+ years with the palpable sense that he could die at any time. In 2010, Mr. Mell reports having



his first bout of panic attacks, and the first of several trips to the emergency room thinking that his life was about to end.

Mr. Mell reports significant mortality fears. He refused a 50th birthday celebration, as he was unwilling to acknowledge the aging process. Mr. Mell reported that the two affairs allowed him to fantasize being 18 again. This is not unusual in those cases in which a confrontation with mortality triggers repressed emotions from earlier traumas that threaten one's existence. I have attached a 2017 article I published in a peer-reviewed professional journal that will introduce you to this phenomenon in greater detail. As you can see from this article, "Existential Issues in Sexual Medicine: The Relation Between Death Anxiety and Hypersexuality," uncharacteristic and uncontrolled sexual behaviors can often be precipitated by a confrontation with mortality and death.

Mr. Mell's behavior was, by all accounts, *uncharacteristic* of any of his prior behaviors. This point is particularly salient when assessing the factors that were driving Mr. Mell's inappropriate sexual behaviors. It is for this reason, as well as his traumatic family history, that I characterize Mr. Mell as a trauma patient as opposed to a sex offender. Given the history of this case, most all sex offender assessment instruments would rate Mr. Mell as a low-risk to re-offend. It is also noteworthy that my impressions mirror very closely those of Richard B. Krueger, M.D., an internationally renowned sex offender expert, who performed an independent evaluation of Mr. Mell. Dr. Krueger reports that Mr. Mell had a relatively high score on the Adverse Childhood Experiences Scale, which suggests significant childhood trauma and adversity. In addition, Dr. Krueger reports that Mr. Mell's sexual history is not suggestive of any sexual deviancy, including pedophilia. Dr. Krueger says, "Mr. Mell's risk according to 5 actuarial instruments of another sexual crime is low or extremely low."

I hope this information will be useful to you in your representation of Mr. Mell. As mentioned earlier, he has done very well in a therapy directed at the treatment of trauma. I believe treatment that continues in this direction will continue to be helpful to him. Please feel free to contact me if I can provide any additional information and/or clarification.

Sincerely yours,

A handwritten signature in black ink, appearing to read 'D. N. Watter', written in a cursive style.

Daniel N. Watter, Ed.D., FAACS
Clinical and Forensic Psychology
Diplomate in Sex Therapy, ABS & AASECT

DNW/ss

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EDUCATION AND TRAINING:

Ed.D. New York University, 1985
M.A. Montclair State College, 1979
B.S.W. University of Alabama, 1978

Certificate in Medical Humanities, Drew University, 2009.

Certificate of Completion, The Kennedy Institute for Ethics Intensive Bioethics Course 34, Georgetown University, 2008.

Child Abuse Case Consultant Project, New Jersey Governor's Task Force on Child Abuse and Neglect, 10/94 – 11/95.

Certificate in Forensic Psychology, New Jersey Academy of Psychology, 1993.

Certificate in Child, Adolescent, and Family Psychotherapy, The Institute for Rational-Emotive Therapy, 1987.

Certificate, The New Jersey Summer School of Alcohol and Drug Studies (Rutgers University), 1982.

Certificate in Rational-Emotive Therapy, The Institute for Rational-Emotive Therapy, 1981.

CERTIFICATION AND LICENSURE:

Fellow: The International Society for the Study of Women's Sexual Health, 2015.

Certified Supervisor, The American Association of Sexuality Educators, Counselors, and Therapists, 2012.

Clinical Fellow, The American Academy of Clinical Sexology, 1991.

Clinical Supervisor, The American Board of Sexology, 1990.

Clinical Psychologist, (N.J. license #35SI00261500), 1989.

Diplomate in Sex Therapy, American Board of Sexology, 1989.

Diplomate in Behavioral Medicine, International Academy of Behavioral Medicine, Counseling, and Psychotherapy, 1988.

Marriage and Family Counselor, (N.J. license #FIO1124), 1986.

Certified Sexologist, The American College of Sexologists, 1985.

Certified Sex Therapist, The American Association of Sexuality Educators, Counselors, and Therapists, 1981.

Certified Sex Educator, The America Association of Sexuality Educators, Counselors, and Therapists, 1979.

WORK EXPERIENCE:

Private practice of psychology, Parsippany, New Jersey, 1981-present.

Clinic Supervisor, Psychological Services Clinic, Rutgers University Graduate School of Applied and Professional Psychology, 2015-present.

Adjunct Professor of Psychology, Drew University, Madison, New Jersey, 2007-2008, 2014.

Adjunct Professor of Psychology, Fairleigh Dickinson University, Madison, New Jersey, 2001-2010.

Adjunct Professor of Psychology and Family Therapy, Seton Hall University, South Orange, New Jersey, 2000-2013.

Clinical Assistant Professor of Psychiatry and Behavioral Medicine, New York College of Osteopathic Medicine, June 1998-present.

Clinical Instructor, Department of OB/GYN, UMDNJ-New Jersey Medical School, Newark, New Jersey, 1988-2013.

Clinical Assistant Professor, Department of Psychiatry, New York Medical College-Lincoln Hospital, New York city, 1987-1993.

Assistant Professor, Department of Health Science, The William Paterson College of New Jersey, Wayne, New Jersey, 1981-1988.

Instructor, New Jersey Summer School for Alcohol and Drug Studies, Rutgers University, 1983.

Consultant, New Jersey Division of Alcoholism, Trenton, New Jersey, 1983.

Director of Sex Education, The Psychological Service Center, Teaneck, New Jersey 1980-1982.

Research Assistant, New York University Department of Health Education, New York City, 1980.

Staff Psychotherapist, The Family Counseling Service of Tuscaloosa, Alabama, 1977-78.

Crisis Counselor, Crisis Intervention Center, Tuscaloosa, Alabama, 1976-78.

COLLEGE COURSES TAUGHT:

Consumer Health
 Counseling: Sexual Issues (graduate)
 Current Health Issues
 Drugs and Health
 Family Life and Sex Education: Methods and Curriculum (graduate)
 Health Counseling
 Human Behavior and Health
 Human Sexuality
 Recent Advances in Sex Therapy (graduate)
 Human Sexuality (graduate)
 Sexual Counseling and Therapy (graduate)

PROFESSIONAL MEMBERSHIPS AND AFFILIATIONS:

American Association of Sexuality Educators, Counselors, and Therapists
 American Psychological Association
 American Society for Bioethics and Humanities
 Association for the Treatment of Sexual Abusers
 International Association for the Treatment of Sexual Offenders
 International Society for Sexual Medicine
 International Society for the Study of Women's Sexual Health
 Morris County Psychological Association
 New Jersey Association of Cognitive-Behavior Therapists
 New Jersey Association for the Treatment of Sexual Abusers
 New Jersey Psychological Association
 Sexual Medicine Society of North America
 Society for Sex Therapy and Research.

PROFESSIONAL OFFICES AND COMMITTEE MEMBERSHIPS:

Chair, Ethics Advisory Committee, The American Association of Sexuality Educators, Counselors, and Therapists. 2018-present.

Member, Special Classification Review Board of the Adult Diagnostic and Treatment Center, 2015-present.

President, The Society for Sex Therapy and Research, 2015-2017.

President-Elect, The Society for Sex Therapy and Research, 2013-2015.

Member, Ethics Committee, The American Association of Sexuality Educators, Counselors, and Therapists, 2012-2014, 2018-present.

Secretary, New Jersey State Board of Psychological Examiners, 2010-2012.

Member, New Jersey State Board of Psychological Examiners, 2009-2012, 2017-present.

Chair, Task Force on Fellows, The American Association of Sexuality Educators, Counselors, and Therapists, 2009-2010.

Member, Ethics Committee, Saint Barnabas Medical Center, Livingston, NJ, 2009-2016.

Secretary/Treasurer, The Society for Sex Therapy and Research, 2008-2013.

Chair Diplomate Certification Committee, The American Association of Sexuality Educators, Counselors, and Therapists, 2007-2012.

Chair, Ethics Committee, New Jersey Psychological Association, 2007-2008.

Co-Chair, Scientific Program, Society for Sex Therapy and Research 32nd Annual Meeting, 2007.

Member, Book Award Committee, The Society for Sex Therapy and Research, 2006-2017.

Member, Sex Therapist Certification Committee, The American Association of Sexuality Educators, Counselors, and Therapists 2006-2012.

Member, Certification Revocation Task Force, The American Association of Sexuality Educators, Counselors, and Therapists, 2006-2007.

Member, Diplomate in Sex Therapy Certification Committee, The American Association of Sexuality Educators, Counselors, and Therapists, 2005-2012.

Member, Ethics Committee, New Jersey Psychological Association, 2002-2008.

Chair, Admissions Committee, The Society for Sex Therapy and Research, 2002-2006.

Chair, New Jersey Psychological Association Program Committee, May 1997-December 2000.

Member, Council on Legislative Affairs, New Jersey Psychological Association, 1992-1994.

Grassroots Coordinator for Legislative District 25, New Jersey Psychological Association, 1991-2000.

Representative to the New Jersey Psychological Association Executive Board, Morris County Psychological Association, 1989-1992.

Member, Interprofessional Relations Committee, New Jersey Psychological Association, 1989-1996.

Member, Program Committee, New Jersey Psychological Association, 1989-2006.

Consulting Editor, THE JOURNAL OF SEX EDUCATION AND THERAPY, 1987-1989.

President, New Jersey Health Education Council, 1986-87.

Vice-President, New Jersey Health Education Council, 1985-86.

Member, Board of Trustees, New Jersey Council on Family Relations, 1985-87.

Recording Secretary, New Jersey Health Education Council, 1984-85.

PUBLICATIONS:

Watter, D.N. (in press). Sexuality and aging: navigating the sexual challenges of aging bodies. In: Y.M. Binik & K.S.K. Hall (eds). PRINCIPLES AND PRACTICE OF SEX THERAPY, 6TH ed. New York: Guilford Press.

Watter, D.N. (In press). "Ethical issues in clinical sexology." In: M. Lew-Starowicz, A. Giraldi, & T. Krueger (eds), PSYCHIATRY AND SEXUAL MEDICINE: A COMPREHENSIVE GUIDE FOR CLINICAL PRACTITIONERS. New York: Springer.

Watter, D.N. (2019). "Supervision in forensic sexology evaluations." In: JC Wadley & R Siegel (eds), THE ART OF SEX THERAPY SUPERVISION. New York: Routledge.

Watter, D.N. (2018). [Review of BECOMING MYSELF: A PSYCHIATRIST'S MEMIOR]. THE JOURNAL OF SEX AND MARITAL THERAPY. 44,423-4.

Watter, D.N. (2018). Protecting my patients' story: beneficent or paternalistic? PATIENT EDUCATION AND COUNSELING. 101(4).

Watter, D.N. (2018). Existential issues in sexual medicine: the relation between death anxiety and hypersexuality. SEXUAL MEDICINE REVIEWS. 6(1).

Watter, D.N. (2017). [Review of THE ENIGMA OF DESIRE: SEX, LONGING, AND BELONGING IN PSYCHOANALYSIS]. THE JOURNAL OF SEX AND MARITAL THERAPY. 42, 660-662, 43, 705-6.

Watter, D.N. (2016). [Review of GALILEO'S MIDDLE FINGER: HERETICS, ACTIVISTS, AND THE SEARCH FOR JUSTICE IN SCIENCE]. THE JOURNAL OF SEX AND MARITAL THERAPY. 42, 660-2.

Perelman, M.A. & Watter, D.N. (2016). "Delayed ejaculation." In S.B. Levine, C.B. Risen, & S.E. Althof (eds.), *THE HANDBOOK OF CLINICAL SEXUALITY FOR MENTAL HEALTH PROFESSIONALS*, 3rd Ed. New York: Routledge.

Watter, D.N. (2016). "Psychological commentary for chapter 12: urologic/clinical evaluation and treatment of disorders of ejaculation." In L.I. Lipshultz, A. Pastuszek, Perelman, M., A. Giraldi, and J. Buster (eds.), *MANAGEMENT OF SEXUAL DYSFUNCTION IN MEN AND WOMEN: AN INTERDISCIPLINARY APPROACH*. New York: Springer.

Watter, D.N. (2016). "Psychological commentary for chapter 13: urologic/clinical evaluation and treatment of orgasmic dysfunction." In L.I. Lipshultz, A. Pastuszek, Perelman, M., A. Giraldi, and J. Buster (eds.), *MANAGEMENT OF SEXUAL DYSFUNCTION IN MEN AND WOMEN: AN INTERDISCIPLINARY APPROACH*. New York: Springer.

Perelman, M.A. and Watter, D.N. (2013). "Delayed Ejaculation." In P.S. Kirana, M.F. Tripodi, Y. Reisman, and H. Porst (eds.), *THE EFS AND ESSM SYLLABUS OF CLINICAL SEXOLOGY*. Amsterdam: Medix Publishers.

Watter, D.N. (2013). "Treatment of male sexual dysfunctions: the clinician's response." In L. Grossman and S. Walfish (eds.), *TRANSLATING RESEARCH INTO PRACTICE: A DESK REFERENCE FOR PRACTICING MENTAL HEALTH PROFESSIONALS*.

Watter, D.N. (2013). [Review of *REHABILITATING SEXUAL OFFENDERS: A STRENGTH-BASED APPROACH*]. *THE JOURNAL OF SEX AND MARITAL THERAPY*. 39, 80-82.

Watter, D.N. (2012). "The medicalization of sex therapy: better living through chemistry?" *THE JOURNAL OF ETHICS IN MENTAL HEALTH*. 7 (Supplement: Sexual behaviours).

Watter, D.N. (2012). "Ethics in sex therapy: a neglected dimension." In Kleinplatz, P.J. *NEW DIMENSIONS IN SEX THERAPY*, 2nd Ed. New York: Routledge.

Watter, D.N. (2007). "Ethical issues in the termination of therapy." *THE NEW JERSEY PSYCHOLOGIST*. 57, 12.

Watter, D.N. and Wahler, L. (2007). "The evaluation and disposition of ethical complaints." *THE NEW JERSEY PSYCHOLOGIST*. 57, 13-14.

Watter, D.N. (2006). "When Ethical Principles Collide: Moral/Ethical Dilemmas in Sexuality." *THE NEW JERSEY PSYCHOLOGIST*. 56, 16-17.

Watter, D.N. (2006). "Forensic sexology versus clinical sexology: some cautionary comments." *SEXUAL AND RELATIONSHIP THERAPY*. 21, 143-148.

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